

Organ Transplant - Self-Care

CCG: C-0038 (CCG)

[Link to Codes](#)

MCG Health
Chronic Care
28th Edition

- Assessment
 - Initial Consideration
 - Condition Knowledge
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Assessment

Initial Consideration

Q1. Patient: What would you like to talk about today? (Check all that apply.) **QM**

- Emergency or urgent problems (eg, social determinants of health, intimate partner violence, suicidal ideation)^[A] Social Determinants of Health Screening Assessment [CCG](#) Intimate Partner Violence [CCG](#) Suicide or Personal Harm Risk [CCG](#)
- Condition information
- Provider coordination (eg, next appointment, treatment clarification) Appointment Scheduling [CCG](#)
- Medications
- Complications
- Monitoring and management
- Other health problems (eg, comorbid conditions, new symptoms)
- Other, specify: _____
- Nothing
- Unknown

Q2. Patient: Do you have any changes since the last time we talked? (Check all that apply.) **QM**

- Adhering to treatment or medication problems Readiness to Change [CCG](#) Medication Adherence [CCG](#)
- Administering medication Home Infusion Management [CCG](#) Injectable Medications [CCG](#)
- Affording medicine or supplies Financial Status and Benefits [CCG](#)
- Aspiration Aspiration Risk Management [CCG](#)
- Behavioral health provider appointment, specify: _____ Follow-Up - Provider Appointment [CCG](#)
- Blood pressure care Blood Pressure Monitoring [CCG](#)
- Blood sugar care Blood Glucose Monitoring [CCG](#) HbA1c Monitoring [CCG](#)
- Blood thinner care Anticoagulation Management [CCG](#)
- Bowel care Bowel Management [CCG](#)
- Bowel changes Bowel Symptoms [CCG](#)
- Caregiver Caregiver Resources Evaluation [CCG](#)
- Chemotherapy Chemotherapy [CCG](#)
- Cognitive impairment Cognitive Impairment [CCG](#) Behavioral Health and Cognition Evaluation with Caregiver [CCG](#)

- Communication or speech problems [Speech and Communication Problems](#)  CCG
- Community resource [Community Resources](#)  CCG
- Dehydration care [Dehydration Monitoring and Management](#)  CCG
- Depression symptoms [Depression Screening, Adult](#)  CCG
- Dialysis care [Dialysis Management](#)  CCG
- Doctor or specialist outpatient appointment, specify: _____ [Follow-Up - Provider Appointment](#)  CCG
- Energy level changes [Energy Level Changes](#)  CCG
- Enteral nutrition care [Enteral Nutrition Management](#)  CCG
- Exercising and staying active [Exercise](#)  CCG
- Fatigue or weakness [Fatigue or Weakness](#)  CCG
- Fluid restriction [Fluid Restriction](#)  CCG
- Functional status changes [Activities of Daily Living](#)  CCG [Instrumental Activities of Daily Living](#)  CCG
- Home infusion [Home Infusion Management](#)  CCG
- Hospital or emergency room visit, specify: _____ [Hospitalization Risk Assessment](#)  CCG [Post-Hospitalization Follow-Up](#)  CCG
- Medication changes [Medication Changes](#)  CCG
- Nebulizer care [Nebulizer Management](#)  CCG
- Neurologic function changes [Neurologic Function Changes](#)  CCG
- Ostomy care [Ostomy Management](#)  CCG
- Oxygen care [Oxygen Management](#)  CCG
- Pain, chronic [Chronic Pain](#)  CCG
- Pain, new or general [Pain Assessment](#)  CCG
- Parenteral nutrition care [Parenteral Nutrition Management](#)  CCG
- Peak flow meter care [Peak Flow Meter Use](#)  CCG
- Peripheral vascular (eg, lower leg redness or swelling) [Peripheral Vascular Problems](#)  CCG
- Physical impairments [Physical Impairments](#)  CCG
- Planning for long-term care needs [Advance Care Planning](#)  CCG
- Procedure, specify: _____ [Follow-Up - Provider Appointment](#)  CCG
- Pulmonary treatments [Pulmonary Treatments](#)  CCG
- Rehabilitation therapy, specify: _____ [Follow-Up - Provider Appointment](#)  CCG
- Respiratory changes [Respiratory Symptoms](#)  CCG
- Safety [Safety](#)  CCG
- Scheduling care [Appointment Scheduling](#)  CCG
- Sleeping problems [Sleep](#)  CCG
- Social support [Social Support](#)  CCG
- Stasis ulcer care [Stasis Ulcer Management](#)  CCG
- Testing (eg, laboratory, imaging), specify: _____ [Follow-Up - Provider Appointment](#)  CCG
- Tracheostomy care [Tracheostomy Management](#)  CCG
- Transportation [Transportation](#)  CCG
- Understanding healthcare needs [Health Literacy](#)  CCG
- Urinary catheter care [Urinary Catheter Management](#)  CCG
- Urinary changes [Urinary Symptoms](#)  CCG
- Venous thromboembolism symptoms [Venous Thromboembolism](#)  CCG
- Ventilator care [Ventilator Management](#)  CCG
- Weight gain [Nutritional Status, Adult](#)  CCG [Nutritional Status, Pediatric](#)  CCG [Nutritional Status, Infant](#)  CCG
- Weight loss [Nutritional Status, Adult](#)  CCG [Nutritional Status, Pediatric](#)  CCG [Nutritional Status, Infant](#)  CCG

- Wound care Wound Management [CCG](#)
- Other, specify: _____
- No
- Unknown Hospitalization Risk Assessment [CCG](#)

Q3. Patient: Have you had a transplant already or are you planning for one?

- Transplant recipient [Q4] [Q5]
- Planning for transplant [Q6] [P1] Transplant Evaluation [CCG](#)
- Unknown [P2]

Q4. Patient: What organ was transplanted? (Check all that apply.)

- Bone marrow Hematopoietic Stem Cell Transplant (Bone Marrow Transplant) [CCG](#)
- Cornea
- Face
- Hand
- Heart Heart Transplant [CCG](#)
- Heart and lung
- Intestine
- Kidney Kidney Transplant [CCG](#)
- Liver Liver Transplant [CCG](#)
- Lung Lung Transplant [CCG](#)
- Pancreas
- Uterus
- Other: _____
- Unknown [P2]

Q5. Patient: How long has it been since your transplant was performed?

- Less than a month
- 1 to 3 months
- 4 to 6 months
- 7 to 12 months
- 13 to 24 months
- More than 2 years
- Unknown [P2]

Q6. Patient: What pretransplant preparation and care still need to be done? (Check all that apply.)

- Cardiology clearance [P1]
- Dental checkup [P1]
- Getting to healthcare appointments [P1]
- Home care needs (eg, supplies) [P1]
- Home support arrangements [P1]
- Pretransplant vaccines, as indicated [P1]
- Psychosocial needs addressed (eg, financial problems, child care needs, living situation changes) [P1]
- Pulmonology clearance [P1]
- Vaccines for children and adults who have close contact, as indicated [P1]
- Other, specify: _____ [P1]
- Unknown [P1]
- None

Condition Knowledge

Q1. Patient: What are the possible complications of an organ transplant? (Check items understood.)

- Avascular necrosis of femoral head
- Bone loss (osteoporosis)
- Cancer (eg, basal cell, Kaposi sarcoma, non-Hodgkin lymphoma, squamous cell, hepatobiliary, in situ uterine cervix)
- Cardiovascular disease
- Depression
- Diabetes
- Fractures due to bone loss
- High blood pressure (hypertension)
- High cholesterol (hypercholesterolemia)

- Infection
- Organ rejection
- Post-transplant cancer
- Primary disease recurrence
- Renal insufficiency
- Sexual problems
- Other, specify: _____
- Nothing [P2]
- Unknown [P2]
- Not all appropriate information known [P2]

Q2. Case Manager: Rate the patient's understanding of the possible complications of organ transplant. **QM**

- 90% or more
- 75% to 89% [P2]
- 50% to 74% [P2]
- Less than 50% [P2]
- Unknown [P2]

Q3. Patient: Do you know what problems can happen after transplant surgery? (Check items understood.)

- Anemia (low red blood cell count)
- Dehydration (dark urine, urinating less, very thirsty, dry mouth, faint or dizzy)
- Heart muscle damage (cardiac toxicity)
- Low platelet count (thrombocytopenia)
- Low white blood cell count (neutropenia)
- Mouth sores (severe mucositis or oral candidiasis)
- Numb, weak, or tingling feeling in arms or legs (neuropathy)
- Post-transplant lymphoproliferative disorder (out-of-control growth of lymph cells)
- Transplanted tissues or cells attack the body (graft vs host disease).
- Upset stomach (nausea), throwing up (vomiting), or loose stools (diarrhea) that does not go away
- Weakness or tiredness
- Other, specify: _____
- Nothing [P2]
- Unknown [P3]
- Not all appropriate information known [P2]

Q4. Patient: How can you care for your transplant after surgery to keep from having problems? (Check items understood.) **QM**

- Adhere to self-care plan
- Ask for help when feelings are overwhelming
- Cancer screenings
- Exercise
- Follow a daily care plan
- Follow an eating (diet) plan (eating only food that is prepared safely)
- Get all follow-up laboratory tests when due
- Instrumental activities of daily living support (eg, help cooking, cleaning)
- Keep all doctor and transplant team appointments
- Keep a vaccination schedule
- Know when to contact the doctor
- Maintain healthy weight
- Make lifestyle changes
- Manage stress
- Manage underlying conditions
- Protect skin when outside (cover skin with clothes and wear sunscreen)
- Safety awareness (fall prevention)
- Seek emergency care for signs of heart or blood vessel problems or stroke
- Set realistic expectations for feeling better, managing symptoms, and recovery
- Stay away from people who have a cold or flu
- Stay away from things that make symptoms worse (avoid areas being vacuumed, stay indoors if mowing or leaf blowing is happening outside)
- Stop using alcohol, tobacco, or drugs
- Take medicines as ordered
- Other, specify: _____ [P2]
- Unknown [P3]

- Nothing [P3]
- Not all appropriate information known [P2]

Q5. Case Manager: Rate the patient's understanding of how to care for the transplant after surgery. **QM**

- 90% or more
- 75% to 89% [P2] [P3]
- 50% to 74% [P2] [P3]
- Less than 50% [P2] [P3]
- Unknown [P2] [P3]

Q6. Patient: What activities would you be able to do if you didn't have problems with your organ transplant? (Check all that apply.) **QM**

- Activities of daily living (ADL) without assistance [P3]
- Cook [P3]
- Drive [P3]
- Exercise [P3]
- Go out with friends [P3]
- Go to events [P3]
- Play sports [P3]
- Play with children or grandchildren [P3]
- Shop [P3]
- Travel [P3]
- Work [P3]
- Other, specify: _____ [P3]
- Unknown [P3]

Provider Information

Q1. Patient: Who manages your organ transplant? **QM**

- Transplant team specialists, names and phone numbers: _____ [Q2]
- Medical home doctor, name and phone number: _____ [Q2]
- Other specialist, name and phone number: _____ [Q2]
- Unknown [P3] [P4]
- Don't have doctor [P2] [P4] [P5]

Q2. Patient: When was your last visit with the doctor or transplant team? **QM**

- 1 month ago or less
- 2 to 3 months ago [P3]
- 4 to 6 months ago [P3]
- 7 to 12 months ago [P3]
- More than 12 months ago [P3]
- Unknown [P3]
- Name of provider seen: _____

Q3. Patient: Are you currently seeing a behavioral health provider (psychiatrist, psychologist, social worker, therapist)?

- Yes, name and phone number and reason: _____
- No
- Unknown [P3]

Q4. Patient: Have you had a dental exam within the last 6 months?

- Yes
- No [P3]
- Unknown [P3]

Q5. Patient: What other specialists have you seen within the last 12 months? (Check all that apply.) **QM**

- Behavioral health provider
- Cardiologist
- Dentist
- Dietitian
- Endocrinologist
- Gastroenterologist
- General surgeon
- Genetic counselor

- Geriatrician
- Gynecologist
- Hematologist
- Hepatologist
- HIV/AIDS
- Infectious disease
- Nephrologist
- Neurologist
- Neurosurgeon
- Obstetrician
- Oncologist
- Ophthalmologist
- Orthopedic surgeon
- Orthopedist
- Otolaryngologist
- Pain management
- Physiatrist
- Plastic surgeon
- Podiatrist
- Pulmonologist
- Rheumatologist
- Urologist
- Other, specify: _____
- None [P2]
- Unknown [P2]

Q6. Patient: How many transplant-related emergency department visits or hospitalizations have you had in the last 12 months? **QM**

- 2 or more [P3] Hospitalization Risk Assessment [CCG](#)
- 1 [P3]
- None
- Other, specify: _____
- Unknown [P3]

Q7. Patient: Do you have a written self-care plan from your doctor on how to care for your organ transplant? [B] **QM**

- Yes [Q8]
- No [P2] [P3]
- Unknown [P2] [P3]

Q8. Patient: What are the components of your organ transplant self-care plan? (Check all that apply.) **QM**

- Action plan for when signs of infection or rejection start
- Blood pressure, pulse, temperature, and weight checks
- Exercise plan
- Infection prevention plan
- Knowing what foods to avoid and how to prepare and cook food safely
- Knowing what medicines you take and why and how to take them
- Managing side effects of medicines
- Medicine use
- Nutrition plan
- Recognizing the signs of complications or rejection and knowing what to do
- Regular follow-up appointments for wellness checks
- When to call the transplant team or doctor and when to get emergency help
- Other, specify: _____ [P2]
- Unknown [P3]
- Not applicable
- Not all appropriate information known [P3]

Q9. Case Manager: Is the patient able to adhere to the self-care plan? **QM**

- Yes, independently
- Yes, with occasional reminders or caregiver support
- Yes, with frequent reminders or caregiver support
- No [P5]
- Unknown [P5]

Medication Information

Q1. Patient: Has any medicine for your transplant been prescribed by the doctor? If so, is it taken as ordered? **QM**

- Yes, and taken as ordered [Q2]
- Yes, but not taken as ordered [Q2] [P6] Medication Adherence  CCG
- No medicine prescribed [P6]
- Unknown [P6]

Q2. Patient: What medicines do you take for your transplant? (Check all that apply.) **QM**

- Corticosteroids (eg, prednisone) [Q3] [P7] [P8]
- Immunosuppressives (eg, azathioprine, basiliximab, belatacept, cyclosporine, mycophenolic acid, mycophenolic mofetil (MMF), sirolimus, tacrolimus) [Q3] [P7] [P9]
- Other, specify: _____ [Q3] [P6] [P7]
- Not applicable [P6]
- Unknown [P7]

Q3. Patient: Are you having side effects from your transplant medicines? (Check all that apply.)

- Constipation [P7]
- Cough [P7]
- Dizziness [P7]
- Fever [P7]
- Headache [P7]
- High blood pressure (hypertension) [P7]
- High blood sugar [P7]
- Kidney function problems [P7]
- Swelling of arms or legs [P7]
- Tingling or numbness, "pins and needles" sensation, or muscle weakness (peripheral neuropathy) [P7]
- Tiredness [P7]
- Trouble breathing [P7]
- Trouble with blood cell counts [P7]
- Upset stomach (nausea) or throwing up (vomiting) [P7]
- Vision problems [P7]
- Wound healing problems [P7]
- Other, specify: _____ [P7]
- None
- Unknown [P7]
- Do not take transplant medicine [P6]

Q4. Patient: What medicine do you take to prevent infections? (Check all that apply.)

- Antifungals
- Antivirals
- Fluoroquinolones
- Macrolides
- Sulfonamides
- Tuberculosis anti-infectives
- Other, specify: _____
- None [P6]
- Unknown [P7]
- Not applicable

Q5. Case Manager: Rate the patient's understanding of the purpose and side effects of the transplant medications. **QM**

- 90% or more
- 75% to 89% [P2] [P7]
- 50% to 74% [P2] [P7]
- Less than 50% [P2] [P7]
- Unknown [P2] [P7]

Complications

Q1. Patient: Are you having any signs of transplant rejection? (Check all that apply.)

- Dyspnea [P3] [P5]
- Fever [P3] [P5]

- Flu-like symptoms, such as chills, upset stomach (nausea), throwing up (vomiting), loose stools (diarrhea), tiredness, headache, dizziness, or body aches and pains [P3] [P5]
- Pain or tenderness over transplant site [P3] [P5]
- Retaining fluids or having sudden weight gain [P3] [P5]
- Sudden rise in blood pressure [P3] [P5]
- Other, specify: _____ [P3] [P5]
- None
- Unknown [P3] [P5]
- Not applicable

Q2. Patient: Are there any new or worsening respiratory problems? (Check all that apply.) **QM**

- Activity intolerance [P10]
- Chest tightness or pressure [P10]
- Coughing [P10]
- Exercise intolerance [P10]
- Phlegm volume increase or color change [P10]
- Positioning increases symptoms (eg, lying flat). [P10]
- Sleep problems [P10]
- Wheezing that you can hear [P10]
- Other, specify: _____ [P10]
- None
- Unknown [P10] [P11]

Q3. Patient: Is there any shortness of breath with the following activities? (Check all that apply.)

- Bathing or grooming [P10]
- Dressing [P10]
- Eating [P10]
- Lying down [P10]
- Sitting [P10]
- Walking across the room [P10]
- Walking around the home [P10]
- Walking up 5 or fewer steps [P10]
- Not short of breath
- Unknown [P10] [P11]

Q4. Patient: When would you get emergency help for breathing problems? (Check items understood.) **QM**

- Breathing heavily and fast
- Breathing medicine (eg, rescue medicine) does not work.
- Cannot say a sentence in one breath
- Chest pain or tightness
- Lips, nose, or nails turn blue or gray.
- Not able to breathe
- Pain when breathing
- Wheezing that quickly gets worse
- Other, specify: _____
- Not all appropriate information known [P11]
- None [P11]
- Unknown [P11]

Q5. Patient: When would you call the doctor for breathing problems? (Check items understood.) **QM**

- Breathing medicine does not work as well as it used to.
- Do not take or cannot take breathing medicine
- Fever over 100.4 degrees F (38 degrees C)
- Have to use breathing medicine more than usual
- Problem with respiratory equipment or medicine
- Start having signs that breathing problems are coming on (eg, short of breath when doing usual activities, or talking causes coughing)
- Went to emergency room because of breathing problems
- Other, specify: _____
- Not all appropriate information known [P11]
- None [P11]

- Unknown [P11]

Q6. Patient: Do you need any help with your respiratory care? (Check all that apply.) **QM**

- Affording medicine or equipment [P11] Financial Status and Benefits [CCG](#)
- Following pulmonary care program [P11] Pulmonary Treatments [CCG](#)
- Following schedule or knowing when to take medicine or use equipment [P10] [P11] Cognitive Impairment [CCG](#) Medication Adherence [CCG](#)
- Knowing how to use equipment or take medicine [P11] Nebulizer Management [CCG](#) Oxygen Management [CCG](#) Tracheostomy Management [CCG](#) Ventilator Management [CCG](#)
- Managing problems of trying to swallow [P10] [P11] Aspiration Risk Management [CCG](#)
- Medicine or equipment does not work well. [P10] [P11]
- Not enough caregiver help [P11] Caregiver Resources Evaluation [CCG](#)
- Other, specify: _____ [P11]
- No
- Unknown [P11]
- Not applicable

Q7. Patient: Do you have any of these bladder problems? (Check all that apply.)

- Feeling like you still need to go after you finish urinating [Q8] [P12] [P13] [P14]
- Frequent urge to urinate [Q8] [P12] [P13] [P14]
- Getting up several times at night to urinate [Q8] [P12] [P14]
- Leaking urine [Q8] [P12] [P14] [P15] [P16]
- Loss of urine control [Q8] [P12] [P14] [P15] [P16]
- Not being able to hold urine long enough to get to bathroom [Q8] [P12] [P14] [P15]
- Pain or burning with urination [Q8] [P12] [P16]
- Pressure in your lower belly [Q8] [P12] [P13] [P16]
- Trouble starting urine stream [Q8] [P12] [P13]
- Unable to urinate [Q8] [P12] [P13] [P14]
- Upset stomach (nausea) or back pain [Q8] [P12] [P16]
- Urinating too often [Q8] [P12] [P14]
- Urine is bright red or has clots in it. [Q8] [P12] [P16]
- Urine is red or cloudy. [Q8] [P12] [P16]
- Other, specify: _____ [Q11] [P12] [P13] [P14] [P16]
- None
- Unknown [P16]

Q8. Patient: Are you taking any medicine for these symptoms? (Check all that apply.)

- Antibiotic for urine infection [Q9]
- Medicine to help you hold your urine: darifenacin, fesoterodine fumarate, mirabegron, onabotulinumtoxinA, oxybutynin, solifenacin, tolterodine, or trospium [Q9]
- Medicine to help you urinate easier: alfuzosin, bethanechol, doxazosin, tamsulosin, or terazosin [Q9]
- Medicine to make your prostate smaller: dutasteride or finasteride [Q9]
- Other, specify: _____ [Q9]
- None
- Unknown
- Not applicable

Q9. Patient: Are you having any side effects from these medicines? (Check all that apply.)

- Abdominal cramps [P13]
- Chest pain [P13]
- Dizziness [P13]
- Dry mouth [P13]
- Dyspnea [P13]
- Erection or ejaculation problems [P13]
- Fainting [P13]
- Hard stools (constipation) [P13]
- Headache [P13]
- Loose stools (diarrhea) [P13]

- Sweating or flushing [P13]
- Swelling of the hands, feet, ankles, or lower legs [P13]
- Upset stomach (nausea) or throwing up (vomiting) [P13]
- Other, specify: _____ [P13]
- None
- Unknown [P16]
- Not applicable

Q10. Patient: When would you get medical care for urinary problems? (Check items understood.)

- Bladder spasms that cannot be relieved with comfort measures
- Blood or stones in urine
- Cloudy, dark yellow, or brown urine
- New or sudden sharp pains in stomach area
- Signs of infection (eg, chills, fever, urge to urinate)
- Signs of overfull bladder (eg, pounding headache, sweating for no reason, cold hands or feet, feeling anxious)
- Sudden change in urine patterns (eg, not making urine)
- Other, specify: _____
- Unknown [P16]
- Not all potential urinary complications understood [P13] [P16]

Q11. Patient: Do you use a urinary catheter? (Check all that apply.)

- Condom catheter (external catheter) Urinary Catheter Management [CCG](#)
- Intermittent (short-term) self-catheter Urinary Catheter Management [CCG](#)
- Indwelling catheter (Foley catheter) Urinary Catheter Management [CCG](#)
- Suprapubic catheter Urinary Catheter Management [CCG](#)
- No
- Unknown

Q12. Patient: Has your mood changed over the past 2 weeks?(1)(2)  PHQ-2 Calculator **QM**

- Yes [Q13]
- No
- Unknown

Q13. Patient: What changes in your mood have you had? (Check all that apply.) **QM**

- Change in usual activity (eg, working excessively, no longer talking to friends, or going on shopping sprees) Depression Screening, Adult [CCG](#)
- Down, hopeless, or depressed Depression Screening, Adult [CCG](#)
- Forgetting a lot or getting angry or frustrated Depression Screening, Adult [CCG](#)
- Little interest or pleasure in doing things Depression Screening, Adult [CCG](#)
- Quick mood changes Depression Screening, Adult [CCG](#)
- Other, specify: _____ Depression Screening, Adult [CCG](#)
- Not applicable

Q14. Case Manager: Rate the patient's understanding of the potential complications of organ transplant. **QM**

- 90% or more
- 75% to 89% [P2] [P3]
- 50% to 74% [P2] [P3]
- Less than 50% [P2] [P3]
- Unknown [P2] [P3]

Monitoring and Management

Q1. Patient: What was your most recent blood pressure reading? **QM**

- Reading was: _____
- Unknown [P17]
- No recent reading [P17]

Q2. Patient: What is your goal blood pressure range? **QM**

- My range is: _____
- Unknown [P17]

Q3. Patient: How often do you check your blood pressure? (Check all that apply.) **QM**

- 2 or more times a day [Q4]
- Once a day [Q4]
- 2 or more times a week [Q4] [P17]
- Once a week or less [Q4] [P17]
- After activity [Q4]
- When it seems like it may be high [Q4]
- Other, specify: _____ [Q4] [P17]
- Unknown [P17]

Q4. Patient: Why do you check your blood pressure? (Check all patient reports.) **QM**

- After a myocardial infarction
- After a stroke
- Coronary artery disease
- Diabetes
- Dizziness or vertigo
- High blood pressure
- Vital sign check
- Other, specify: _____ [P17]
- Unknown [P17]
- Not all appropriate reasons to check blood pressure reported [P17]

Q5. Patient: Do you measure your blood pressure at home?

- Yes
- No [P17]
- Unknown [P17]

Q6. Case Manager: Is patient's blood pressure within goal range? **QM**

- Yes
- No [P17]
- Unknown [P17]

Q7. Patient: How does your ability to do activities compare with that of 6 months ago? **QM**

- Stable
- Improved
- Declined [P18] [P19] Activities of Daily Living [CCG](#)
- Unknown [P19]

Q8. Patient: Are you having any problems managing care due to physical impairments? (Check all that apply.) **QM**

- Doing activities of daily living (eg, bathing, dressing, or eating) [P18] [P19] Activities of Daily Living [CCG](#)
- Doing instrumental activities of daily living (eg, shopping, cooking, or housework) [P18] [P19] Instrumental Activities of Daily Living [CCG](#)
- Equipment problems [P18] [P19] Assistive Devices [CCG](#)
- Getting caregiver help [P18] [P19] Caregiver Resources Evaluation [CCG](#)
- Getting transportation [P19] Transportation [CCG](#)
- Other, specify: _____ [P18] [P19]
- No problems
- Unknown [P18] [P19]

Q9. Patient: Have there been any falls? **QM**

- Yes [P18]
- No
- Unknown [P18]

Q10. Patient: Have you had rehabilitation in the last 6 months? (Check all that apply.) **QM**

- Cognitive therapy
- Occupational therapy
- Physical therapy
- Speech therapy (including for swallowing evaluation and treatment of swallowing disorder)
- Other, specify: _____
- All appropriate therapy received

- None [P19]
- Unknown [P19]

Q11. Patient: How do you get around? (Check all that apply.) **QM**

- Walk without help (independent without device use) [P18] Safety  CCG
- Walk with help (eg, use of cane or brace) [P18] [P19] Safety  CCG
- Walk at home but use a wheelchair when going out [Q12] [P18] [P19] Safety  CCG
- Walk with someone's help [P18] [P19] Safety  CCG
- Walk but tire easily or are unsteady [P18] [P19] Safety  CCG
- Wheelchair-bound [Q12] [P18] [P19] Safety  CCG
- Bed-bound [Q13] [P18] [P19] Safety  CCG
- Other, specify: _____ [P19] Safety  CCG
- Unknown [P19] Safety  CCG

Q12. Patient: Are there any problems managing wheelchair tasks? (Check all that apply.) **QM**

- Crossing streets [P19]
- Equipment problems [P18] [P19]
- Folding and opening wheelchair (manual only) [P18] [P19]
- Maneuvering through tight spaces such as hallways, over uneven or irregular terrain, over curbs, and up ramps [P18] [P19]
- Opening, closing, and going through doors [P19]
- Operating control system for power wheelchair [P18] [P19]
- Performing wheelies (manual only) [P18] [P19]
- Reaching for objects above and below [P18] [P19]
- Transfers (eg, chair, bed) [P18] [P19]
- Using basic skills such as braking, using armrests and footrests, and self-propulsion [P18] [P19]
- Other, specify: _____ [P18] [P19]
- No problems
- Unknown [P18] [P19]
- Not applicable

Q13. Patient: Are there any problems managing tasks while bed-bound? (Check all that apply.) **QM**

- Moving in bed [P19]
- Picking up objects [P18] [P19]
- Reaching for objects above and below [P18] [P19]
- Other, specify: _____ [P18] [P19]
- No problems
- Unknown [P18] [P19]
- Not applicable

Q14. Patient: Do you sometimes have problems thinking or remembering? If so, what problems do you have? (Check all that apply.) **QM**

- Activities of daily living tasks (eg, bathing, oral hygiene) [Q15] [P20]
- Appointment forgetfulness [Q15] [P20]
- Concentrating during conversations [Q15] [P20]
- Date (ie, time, month, year) [Q15] [P20]
- Decisiveness [Q15] [P20]
- Impulsive [Q15] [P20]
- Instrumental activities of daily living tasks (eg, grocery shopping, paying bills) [Q15] [P20]
- Location (ie, getting lost) [Q15] [P20]
- Medicine forgetfulness [Q15] [P20]
- Names or people [Q15] [P20]
- Past events, recent [Q15] [P20]
- Problem solving [Q15] [P20]
- Safety steps (eg, locking doors, turning off stove) [Q15] [P20]
- Train of thought [Q15] [P20]
- No
- Unknown [Q15] [P20] Behavioral Health and Cognition Evaluation with Caregiver  CCG

Q15. Patient: How does your current status of thinking or remembering compare with that of 6 months ago?

- About the same
- Better
- Worse [B1] [P20] Behavioral Health and Cognition Evaluation with Caregiver [CCG Preliminary Depression Screening](#) [CCG](#)
- Unknown [B1] [P20] Behavioral Health and Cognition Evaluation with Caregiver [CCG Preliminary Depression Screening](#) [CCG](#)

Q16. Patient: Do you have problems with day-to-day life because of trouble thinking or remembering? **QM**

- Yes [Q17] [B1] [P20] Behavioral Health and Cognition Evaluation with Caregiver [CCG Preliminary Depression Screening](#) [CCG](#)
- No
- Unknown [Q17] [B1] [P20] Behavioral Health and Cognition Evaluation with Caregiver [CCG Preliminary Depression Screening](#) [CCG](#)

Q17. Patient: Is there someone who helps you with your self-care and daily responsibilities? If so, who? **QM**

- Yes, specify: _____ Caregiver Resources Evaluation [CCG](#)
- No [B1] [P20]
- Unknown [B1] [P20] Behavioral Health and Cognition Evaluation with Caregiver [CCG Preliminary Depression Screening](#) [CCG](#)

Q18. Patient: How often are you supposed to weigh? **QM**

- Every day [Q19]
- Once a week
- Once a month
- Other, specify: _____
- No specific time [P21]
- Unknown [P21]

Q19. Patient: Do you weigh every day?

- Yes
- No, because: _____ [P21]
- Unknown [P21]

Q20. Patient: Do you keep a diary of weights?

- Yes
- No [P21]
- Unknown [P21]

Q21. Patient: Do you know how much weight gain or loss should trigger an action on your part? **QM**

- Yes
- No [P21]
- Unknown [P21]

Q22. Patient: Do you have any pain? **QM**

- Yes [P3] [P5] Pain Assessment [CCG](#)
- No
- Unknown [P2]

Q23. Patient: Do you get regular exercise? **QM**

- Yes Exercise [CCG](#)
- No [P3]
- Unknown [P3]

Q24. Patient: Are you trying to gain or lose weight?

- Trying to gain weight [P3] [P5] Nutritional Status, Adult [CCG Nutritional Management to Gain Weight](#) [CCG](#)
- Trying to lose weight Nutritional Status, Adult [CCG Nutritional Management to Lose Weight](#) [CCG](#)
- Neither
- Unknown [P2]

Q25. Patient: Do you have any risk factors for infection? (Check all that apply.)

- Changing of diapers [P3]
- Crowded household [P3]
- Exposure to animals (eg, cats, dogs, reptiles, birds, or farm animals) [P3]

- Exposure to person with known infectious disease (eg, tuberculosis or hepatitis) [P3]
- Indwelling vascular access [P3]
- Live in group home [P3]
- Live in transient housing or shelter [P3]
- Small children in household [P3]
- Unsafe sexual practices (eg, multiple partners or unprotected contact) [P3]
- Other, specify: _____ [P3]
- No risk factors or exposures
- Unknown [P3]
- Not applicable

Q26. Patient: Are you being treated for a serious infection? If so, how?

- Yes, specify: _____ [P7] Medication Adherence  CCG
- No
- Unknown [P7]

Q27. Patient: What steps do you take to cut down your risk of getting an infection? (Check all that apply.)

- Do not eat food that has been left out
- Do not share personal items (eg, razors, bath towels)
- Prepare and cook food safely
- Safe sexual practices
- Safe travel precautions
- Stay away from bodily secretions
- Stay away from children and adults who have not received appropriate vaccinations
- Stay away from cold-mist humidifiers
- Stay away from construction or renovation areas
- Stay away from contact with pets and other animals
- Stay away from crowds and people who are sick with colds or flu
- Stay away from exotic animals
- Stay away from high-risk foods (eg, raw fish or meat, unclean water, unpasteurized dairy products)
- Wash hands a lot
- Wear gloves when working with hands
- Other, specify: _____
- None [P3]
- Unknown [P3]
- Not all appropriate information known [P3]

Q28. Patient: Do you need any of the following preventive services? (Check all that apply.)

- Cholesterol check [P3] Cholesterol Monitoring  CCG
- Colonoscopy [P3]
- Dual-energy x-ray absorptiometry (DXA) scan (osteoporosis) [P3]
- Eye examination [P3]
- Mammogram [P3]
- Pap test [P3]
- Prostate digital examination and prostate-specific antigen (PSA) test [P3]
- Skin examination [P3]
- Vaccinations [P3] Vaccinations, Adult  CCG
- None
- Unknown [P3]

Q29. Patient: Are there any medical treatments that are part of your organ transplant self-care plan? (Check all that apply.)

- Blood pressure monitoring [P3] [P5] Blood Pressure Monitoring  CCG
- Blood sugar monitoring [P3] [P5] Blood Glucose Monitoring  CCG
- Dialysis [P3] [P5] Dialysis Management  CCG
- Home IV infusion [P3] [P5] Home Infusion Management  CCG
- IV feedings (parenteral nutrition) [P3] [P5] Parenteral Nutrition Management  CCG
- Medical home needs [P3] [P5] Medical Home Coordination  CCG
- Nebulizer treatments [P3] [P5] Nebulizer Management  CCG
- Oxygen [P3] [P5] Oxygen Management  CCG

- Pulmonary treatments [P3] [P5] Pulmonary Treatments [CCG](#)
- Tube feedings (enteral nutrition) [P3] [P5] Enteral Nutrition Management [CCG](#)
- Urinary catheter care [P3] [P5] Urinary Catheter Management [CCG](#)
- Wound management [P3] [P5] Wound Management [CCG](#)
- Other, specify: _____ [P3] [P5]
- None
- Unknown [P2]

Q30. Patient: Are you having any problems with your transplant self-care needs? (Check all that apply.) **QM**

- Affording medicine or supplies [P3] [P5] Financial Status and Benefits [CCG](#)
- Behavior indicating cognitive impairment [P3] [P5] Cognitive Impairment [CCG](#) Behavioral Health and Cognition Evaluation with Caregiver [CCG](#)
- Blood pressure care [P3] [P5] Blood Pressure Monitoring [CCG](#)
- Blood sugar care [P3] [P5] Blood Glucose Monitoring [CCG](#)
- Bowel program [P3] [P5] Bowel Management [CCG](#)
- Caregiver help needed [P3] [P5] Caregiver Resources Evaluation [CCG](#)
- Community resource information needed [P3] [P5] Community Resources [CCG](#)
- Exercising and staying active [P3] [P5] Exercise [CCG](#)
- Fatigue or weakness [P3] [P5] Fatigue or Weakness [CCG](#)
- Medical home needs [P3] [P5] Medical Home Coordination [CCG](#)
- Planning for long-term care needs [P3] [P5] Advance Care Planning [CCG](#) Palliative or End-of-Life Evaluation [CCG](#)
- Safety in the home [P3] [P5] Safety [CCG](#)
- Skin care management [P3] [P5] Pressure Injury Monitoring and Management [CCG](#)
- Sleeping problems [P3] [P5] Sleep [CCG](#)
- Social support [P3] [P5] Social Support [CCG](#)
- Taking enough food or fluids [P3] [P5] Dehydration Monitoring and Management [CCG](#) Nutritional Management to Gain Weight [CCG](#)
- Taking medicine as prescribed [P3] [P5] Medication Adherence [CCG](#)
- Transitioning care from pediatrics to adult [P3] [P5]
- Transportation for care [P3] [P5] Transportation [CCG](#)
- Trouble gaining weight [P3] [P5] Nutritional Status, Adult [CCG](#) Nutritional Management to Gain Weight [CCG](#)
- Trouble losing weight [P3] [P5] Nutritional Status, Adult [CCG](#) Nutritional Management to Lose Weight [CCG](#)
- Trouble making lifestyle changes [P3] [P5] Readiness to Change [CCG](#)
- Trouble understanding how to do self-care [P3] [P5] Health Literacy [CCG](#)
- Other, specify: _____ [P3] [P5]
- No
- Unknown [P3]

Q31. Case Manager: Rate the patient's understanding of the monitoring and management of the transplant. **QM**

- 90% or more
- 75% to 89% [P2] [P3]
- 50% to 74% [P2] [P3]
- Less than 50% [P2] [P3]
- Unknown [P2] [P3]

Other Health Considerations

Q1. Patient: Do you use tobacco, alcohol, or drugs? (Check all that apply.) **QM**

- Yes, tobacco use Tobacco Use [CCG](#)
- Yes, alcohol use Alcohol Use [CCG](#)
- Yes, drug use Substance Use [CCG](#)
- No
- Unknown Tobacco Use [CCG](#) Alcohol Use [CCG](#) Substance Use [CCG](#)

Q2. Patient: Are you pregnant or planning pregnancy?

- Yes, pregnant [P3] [P5] [High-Risk Pregnancy - Self-Care](#)  CCG
- Yes, planning pregnancy [P3]
- No
- Unknown [P2]

Q3. Patient: Do any children and adults in close contact with you need any vaccinations? (Check all that apply.)

- Coronavirus disease 2019 (COVID-19)
- *Haemophilus influenza* type b [P3]
- Hepatitis A [P3]
- Hepatitis B [P3]
- Measles, mumps, or rubella (MMR) [P3]
- Varicella (chickenpox) [P3]
- Other, specify: _____ [P3]
- No
- Unknown [P3]
- Not applicable

Q4. Patient: Do you have other conditions that might cause problems with your transplant care? (Check all that apply.) **QM**

- Alcohol or substance misuse [P3] [P5] [Substance-Related Disorders - Self-Care](#)  CCG [Substance Use](#)  CCG
- Cancer [P3] [P5] [Cancer - Self-Care](#)  CCG
- Cardiovascular disease [P3] [P5] [Coronary Artery Disease - Self-Care](#)  CCG [Heart Failure - Self-Care](#)  CCG
- Chronic obstructive pulmonary disease (COPD) [P3] [P5] [Chronic Obstructive Pulmonary Disease - Self-Care](#)  CCG
- Depression [P3] [P5] [Depression - Self-Care](#)  CCG
- Diabetes [P3] [P5] [Diabetes, Adult - Self-Care](#)  CCG
- Gastroesophageal reflux [P3] [P5]
- Gout [P3] [P5] [Arthritis - Self-Care](#)  CCG
- High blood pressure [P3] [P5] [Hypertension - Self-Care](#)  CCG
- High cholesterol [P3] [P5] [High Cholesterol - Self-Care](#)  CCG
- HIV/AIDS [P3] [P5] [HIV/AIDS - Self-Care](#)  CCG
- Kidney disease [P3] [P5] [Kidney Disease - Self-Care](#)  CCG
- Liver disease [P3] [P5] [Liver Disease - Self-Care](#)  CCG
- Obesity or overweight [P3] [P5] [Obesity, Adult - Self-Care](#)  CCG [Obesity, Pediatric - Self-Care](#)  CCG
- Osteoporosis [P3] [P5] [Osteoporosis - Self-Care](#)  CCG
- Physical inactivity [P3] [P5] [Exercise](#)  CCG
- Posttraumatic stress disorder (PTSD) [P3] [P5] [Posttraumatic Stress Disorder](#)  CCG
- Pregnancy [P3] [P5] [High-Risk Pregnancy - Self-Care](#)  CCG
- Other, specify: _____ [P3] [P5]
- No
- Unknown [P3]

Patient Understanding Evaluation

Q1. Case Manager: Does the patient understand all the complications of an organ transplant?

- Yes
- No [P5]
- Unknown [P5]

Q2. Case Manager: Does the patient understand the signs of problems after transplant surgery?

- Yes
- No [P3]
- Unknown [P3]

Q3. Case Manager: Does the patient understand how to care for the transplant? **QM**

- Yes
- No [P1] [P3]
- Unknown [P1] [P3]

Q4. Case Manager: Does the patient know when and where to get help for problems?

- Yes
- No [P2] [P5]
- Unknown [P5]

Q5. Case Manager: Does the patient have the ability to understand and follow the self-care plan? **QM**

- Yes
- No [P5]
- Unknown [P5]

Q6. Case Manager: Does the patient understand the purpose and side effects of their transplant medication?

- Yes
- No [P7]
- Unknown [P7]

Q7. Case Manager: Is the patient having trouble coping with their transplant? **QM**

- Yes [P5]
- No
- Unknown [P5]

Q8. Case Manager: Does the patient agree to education? **QM**

- Yes
- No, explain why: _____ [P5] Readiness to Change  CCG
- Unknown [P5]

Quality Measures

- Disease Management Quality Measures:
 - Information provided from this question may be requested by various accrediting organizations.
- HEDIS Measures include(3):
 - Tobacco use is assessed and addressed.
 - Readmission risk is assessed and addressed.
 - Alcohol and substance misuse is assessed and addressed.

Plan of Care

Barriers

B1. Cognitive impairment present

Problems

P1. Pretransplant information needed

- **Goal:** Pretransplant planning better understood(4)(5)(6)
 - **Interventions:**
 - **ASSIST:** Identifying patient and caregiver's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with transplant team for screening
 - **ASSIST:** Identifying potential donors, if appropriate
 - **ASSIST:** Navigation of health system and communication between multiple providers
 - **ASSIST:** Completing record of all medical care
 - **ASSIST:** Support system (eg, family, friends) identification, enlistment, and education
 - **ASSIST:** Identifying lodging for patient and caregiver if travel needed
 - **ASSIST:** Appointment scheduling with provider for discussion on treatment options
 - **EDUCATE:** Pretransplant preparations include:
 - Transplant requirements, if needed (eg, smoking cessation, vaccinations up to date)
 - Home support arrangements
 - Home care needs (eg, supplies)
 - Psychosocial preparation (eg, loss of work time, child care needs, living situation changes)(7)
 - Dental checkup
 - Testing (eg, blood typing, preoperative chest x-ray)
 - Insurance coverage for procedure and medications
 - Creation of list of phone numbers for team members available 24 hours a day, 7 days a week

- Transplant procedure
- Post-transplant expectations
- Lodging arrangements for patient and caregiver if travel needed
- Transportation for pretransplant and post-transplant care needs
- Vaccines for children and adults who have close contact, as indicated
- **EDUCATE:** What to do when organ disease symptoms worsen
- **EDUCATE:** Alternative transplant treatment options may include:
 - Medication management
 - Surgical interventions
 - End-of-life or palliative care
- **COORDINATE:** Pretransplant completion of health screenings (ophthalmologist, dentist, ongoing age-appropriate and gender-appropriate cancer screening), if indicated
- **COORDINATE:** Specialist referral for comorbidities, if indicated
- **COORDINATE:** Transplant center referral, if indicated
- **COORDINATE:** Laboratory testing, if indicated
- **COORDINATE:** Outpatient imaging, tests, or procedures, if indicated
- **COORDINATE:** Community services referral, as needed
- **COORDINATE:** Social worker referral, as needed(8)(9)
- **COORDINATE:** Support group referral, as needed(7)
- **COORDINATE:** Home care referral, if indicated(10)
- **COORDINATE:** Rehabilitation therapy referral, if indicated(11)
- **COORDINATE:** Tobacco use treatment, as needed(9)
- **COORDINATE:** Care team meeting, if needed(12)(13)
- **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
- **COORDINATE:** Follow-up to evaluate pretransplant preparation understanding, referral status, and need for further assistance(16)(17)
- **SEND:** Before your transplant  (Spanish)
- **SEND:** Vaccines before your transplant  (Spanish)
- **SEND:** Advance care planning  (Spanish)
- **SEND:** Be ready for your doctor visit - End-of-life care  (Spanish)
- **SEND:** Be ready with questions for your doctor - End-of-life care  (Spanish)
- **SEND:** Hospice care  (Spanish)

P2. Transplant information needed

- **Goal:** Organ transplant better understood(4)(5)(6) 
- **Interventions:**
 - **ASSIST:** Identifying patient and caregiver's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling
 - **EDUCATE: Definition:**
 - Solid organ transplant removes a diseased organ and replaces it with a cadaver or living donor organ.
 - **EDUCATE: Reasons for transplant(18):**
 - Cancer
 - Diabetes (pancreas transplant)(19)
 - Disease recurrence
 - Life-threatening, end-stage organ failure
 - Rejection of a previously transplanted organ
 - **EDUCATE: Treatment(20):**
 - Lifestyle changes (eg, nutrition, exercise, tobacco cessation)
 - Self-care activities (eg, diet, activity, medication)
 - Laboratory test monitoring
 - Depends on specific clinical condition and prognosis
 - Immunotherapy
 - Medication management
 - Fluid management
 - Comorbid condition management
 - Medications for pre-existing conditions may need to be changed (type or dose).
 - Symptom management
 - **EDUCATE: Treatment goals(18):**

- Complication and progression management
- Symptom frequency and severity reduced
- Quality of life improvement
- **EDUCATE: Notify transplant team of symptoms of rejection** (common symptoms of rejection for all solid organ transplants)(18):
 - Flu-like symptoms
 - Irritability (in children)
 - Fever over 100.4 degrees F (38 degrees C)
 - Weight gain or loss without reason
 - Fluid buildup (eg, edema, tight clothes, dizziness)
 - Heart rate changes
 - Urinary problems (eg, decreased output)
- **EDUCATE:** All organ transplants have increased risk of organ failure.
- **EDUCATE: Resources:**
 - Transplant Living. United Network for Organ Sharing (UNOS): www.transplantliving.org
 - MedlinePlus, a service of the U.S. National Library of Medicine and National Institutes of Health: www.medlineplus.gov
 - Smokefree. National Institutes of Health: www.smokefree.gov
- **COORDINATE:** Dietitian referral, if indicated
- **COORDINATE:** Home care referral, if indicated(10)
- **COORDINATE:** Laboratory testing, if indicated
- **COORDINATE:** Care team meeting, if needed(12)(13)
- **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
- **COORDINATE:** Follow-up to evaluate organ transplant understanding, referral status, and need for further assistance(16)(17)
- **SEND:** Your care plan - Organ transplant  (Spanish)
- **SEND:** What you need to know - Organ transplant  (Spanish)
- **SEND:** When to get help - Organ transplant  (Spanish)
- **SEND:** Daily care for your condition - Organ transplant  (Spanish)
- **SEND:** Be ready for your doctor visit - Organ transplant  (Spanish)
- **SEND:** Be ready with questions for your doctor - Organ transplant  (Spanish)
- **Goal:** Intestinal transplant information better understood(4)(5)(6)
 - **Interventions:**
 - **ASSIST:** Identifying patient and caregiver's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling
 - **EDUCATE: Definition**(21):
 - Intestinal transplant replaces the small intestine due to intestinal failure caused by injury or disease.
 - **EDUCATE: Transplant types**(21):
 - Isolated intestinal transplant: small bowel transplant
 - Combined liver-intestine transplant: small bowel and liver transplant
 - Multivisceral transplant: small bowel plus other abdominal organs such as the stomach, liver, spleen, or pancreas with or without the liver
 - Modified multivisceral transplant: small bowel plus other visceral organs without the liver
 - **EDUCATE: Reasons for transplant in children**(22):
 - Short bowel problems (eg, congenital disorders, volvulus, necrotizing enterocolitis)
 - Intestinal dysmotility (eg, pseudo-obstruction, Hirschsprung disease)
 - Enterocyte dysfunction (eg, microvillus inclusion disease, Crohn disease)
 - Tumors
 - **EDUCATE: Reasons for transplant in adults**(21):
 - Volvulus
 - Pseudo-obstruction
 - Ischemia
 - Crohn disease
 - Desmoid tumor
 - Gardner disease
 - Trauma
 - **EDUCATE: Treatment goal:** Freedom from total parenteral nutrition (TPN)(21)(22)

- **EDUCATE: Complications** (in addition to those with solid organ transplant)(21):
 - Infection
 - Rejection
 - Fluid and electrolyte imbalance
- **EDUCATE: Notify transplant team of symptoms of rejection** (in addition to those with solid organ transplant)(21)(22):
 - Abdominal pain and distention
 - GI bleeding
 - Nausea and vomiting
 - Increased stomal output or dusky-colored stoma
 - Septic shock
- **EDUCATE:** Enterostomal care and management, if appropriate
- **COORDINATE:** Dietitian referral, if indicated
- **COORDINATE:** Home care referral, if indicated(10)
- **COORDINATE:** Rehabilitation therapy referral, if indicated[C]
- **COORDINATE:** Wound, ostomy, and continence nurse (WOCN) referral, if indicated
- **COORDINATE:** Laboratory testing, if indicated
- **COORDINATE:** Ostomy supplies, as needed
- **COORDINATE:** Care team meeting, if needed(12)(13)
- **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
- **COORDINATE:** Follow-up to evaluate intestinal transplant understanding, referral status, and need for further assistance(16)(17)
- **SEND:** Ileostomy care - Caring for your stoma (Illustrated)  (Spanish)
- **SEND:** Ileostomy care - Changing your ileostomy pouch (Illustrated)  (Spanish)
- **SEND:** Ileostomy care - Emptying your ileostomy pouch (Illustrated)  (Spanish)
- **Goal:** Pancreas transplant information better understood(4)(5)(6)
 - **Interventions:**
 - **ASSIST:** Identifying patient and caregiver's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling
 - **EDUCATE: Reasons for transplant**(19):
 - Diabetes with poor metabolic control
 - Pancreatic exocrine insufficiency
 - **EDUCATE: Complications** (in addition to those with solid organ transplant):
 - Bleeding
 - High blood sugar counts
 - Kidney failure
 - Lymphoproliferative disease
 - Pancreatitis
 - Vascular thrombosis
 - **EDUCATE: Notify transplant team of symptoms of rejection** (in addition to those with solid organ transplant)(18):
 - Glucose control changes
 - Unexplained leukocytosis
 - Weight increase, sudden
 - Dyspnea
 - Graft swelling and pain
 - **COORDINATE:** Dietitian referral, if indicated
 - **COORDINATE:** Home care referral, if indicated(10)
 - **COORDINATE:** Laboratory testing, if indicated
 - **COORDINATE:** Care team meeting, if needed(12)(13)
 - **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
 - **COORDINATE:** Follow-up to evaluate pancreas transplant understanding, referral status, and need for further assistance(16)(17)

P3. Transplant self-care needed

- **Goal:** Transplant self-care plan developed, implemented, and maintained(4)(5)(23)(24) 
- **Interventions:**
 - **ASSIST:** Identifying patient and caregiver's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)

- **ASSIST:** Appointment scheduling
- **ASSIST:** Self-care plan creation between patient and provider (eg, goals, management, treating symptoms, getting help)
- **ASSIST:** Obtaining a medical alert bracelet or necklace
- **ASSIST:** Communication between multiple providers(25)
- **EDUCATE: Self-care plan information(18):**
 - Action plan (eg, rejection, infection, high blood sugars)
 - Daily care (eg, caring for transplant, staying healthy)
 - Medication purpose, use, effects, and considerations (eg, timing before meals or exercise)
 - Weight monitoring and management
 - Health checks, diary, and action plans (eg, blood pressure, pulse, temperature, and weights). Take diary to all provider appointments.
 - Diet considerations (eg, food safety, balanced diet)
 - Energy conservation measures (eg, rest periods, sit often, healthy sleeping habits)
 - Functional changes care (eg, adaptations and support for ADL and IADL)
 - Exercise program within provider's recommendations(11)(26)N
 - Behavioral health monitoring and care (eg, depression monitoring and management)
 - Pain management plan
 - Stress management (eg, relaxation and coping techniques)
 - Infection prevention measures (eg, hand hygiene, avoid crowds)
 - Laboratory monitoring, as needed
 - Imaging, tests, or procedures, if indicated(27)
 - Medical alert bracelet/necklace, obtain and wear
 - Skin care management (eg, limiting sun exposure when outdoors by using SPF-30 or higher sunscreen; wearing protective clothing, brimmed hat, sunglasses)
 - Wellness checkups (eg, vaccinations, screenings)
 - Preconception counseling, if indicated(18)
 - Tobacco cessation plan
 - Support group participation(7)
- **EDUCATE: When to get emergency help:**
 - Heart attack (eg, persistent chest pain; pain radiating to the arms, shoulders, neck, back, and/or jaw; cool clammy skin)(28)
 - Stroke (eg, face drooping, arm weakness, speech problems, bad headache, vision changes, loss of balance or coordination)(29)
 - Respiratory impairment (eg, gasping for air, choking, cyanosis, chest pain, severe dyspnea)(30)
 - Cough with production of colored sputum
 - Heartbeat irregular, new
 - Headache (eg, sudden severe, unrelieved by analgesics)
 - Deep venous thrombosis (eg, redness, swelling, and tenderness of leg)
 - Pain (eg, sudden, severe, unrelieved by analgesics)
 - Fever greater than 101 degrees F (38.3 degrees C)
 - Bleeding complication (eg, does not stop after 15 minutes, hematemesis, melena)
 - Fatigue, extreme
 - Suicidal or homicidal actions or plans(31)
- **EDUCATE: When to call the provider:**
 - Blood glucose outside of provider's parameters
 - Hyperglycemia (eg, elevated blood glucose, nausea, frequent urination)(32)
 - Cognitive changes (eg, thinking problems, forgetfulness worsens)(18)
 - Infection (eg, fever at or over 100.4 degrees F (38 degrees C) or chills, flu-like symptoms, dizziness, stuffy or runny nose, sore throat or cough, enlarged lymph nodes, or wound drainage)(27)
 - Wound problems (eg, red, swollen, purulent discharge)
 - Fluid buildup (eg, edema, tight clothes, dizziness)(27)
 - Blood pressure outside of provider's parameters(9)
 - Pain changes (eg, new or worsening, unresolved with treatment)
 - Skin problems (eg, new or unusual rash, mole, lesion, jaundice)(18)
 - Mucositis or oral candidiasis
 - Weight gain or loss without reason
 - Bowel or bladder dysfunction
 - Depression (eg, feeling sad, hopeless, overwhelmed)(1)(33)
 - Insomnia
 - Fatigue, unrelenting
 - Mumps, measles, chickenpox, or shingles exposure(34)
 - Self-care changes (eg, ADL or IADL problems)

- Comorbid condition exacerbation
- Substance misuse (eg, increased use, use interferes with ADL/IADL)
- Falls
- Medication problems (eg, side effects, access, administration)
- Visual changes, new-onset(27)
- Gastrointestinal distress (eg, nausea, vomiting, diarrhea, cramping)(27)
- Pregnant or planning to get pregnant
- Mood changes (eg, depression, anger, agitation)
- Rejection (eg, flu-like symptoms, irritability (in children), fever, weight changes, swelling, changes in heart rate, or urinating less often)(18)
- Self-care plan problems (eg, unable to follow, condition exacerbation)
- **COORDINATE:** Behavioral health services referral, if indicated(18)
- **COORDINATE:** Dietitian referral, if indicated
- **COORDINATE:** Home care referral, if indicated(10)
- **COORDINATE:** Specialist referral for comorbidities, if indicated
- **COORDINATE:** Laboratory testing, if indicated
- **COORDINATE:** Outpatient imaging, tests, or procedures, if indicated
- **COORDINATE:** Social worker referral, as needed(8)
- **COORDINATE:** Support group referral, as needed(7)
- **COORDINATE:** Vocational rehabilitation referral, as needed
- **COORDINATE:** Medical equipment and supplies, if indicated
- **COORDINATE:** Tobacco use treatment, as needed
- **COORDINATE:** Care team meeting, if needed(12)(13)
- **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
- **COORDINATE:** Follow-up contact with patient to evaluate understanding and adherence to self-care plan, referral status, and need for further assistance(16)(17)
- **SEND:** Your care plan - Organ transplant  (Spanish)
- **SEND:** What you need to know - Organ transplant  (Spanish)
- **SEND:** When to get help - Organ transplant  (Spanish)
- **SEND:** Daily care for your condition - Organ transplant  (Spanish)
- **SEND:** Be ready for your doctor visit - Organ transplant  (Spanish)
- **SEND:** Be ready with questions for your doctor - Organ transplant  (Spanish)
- **SEND:** Dry mouth  (Spanish)
- **SEND:** Help with sleeping or insomnia  (Spanish)
- **SEND:** How to manage your pain  (Spanish)
- **SEND:** How to stop smoking  (Spanish)
- **SEND:** Ways to save energy  (Spanish)
- **SEND:** Vaccines after your transplant  (Spanish)
- **SEND:** Tips for good mental health  (Spanish)
- **SEND:** Important healthcare phone numbers  (Spanish)
- **SEND:** What you need to know - Depression  (Spanish)
- **SEND:** Symptoms diary (Illustrated)  (Spanish)
- **SEND:** Vital signs diary (Illustrated)  (Spanish)
- **SEND:** Using an ear thermometer for your child (Illustrated)  (Spanish)
- **SEND:** How to check your blood pressure (Illustrated)  (Spanish)
- **SEND:** How to measure your heart rate (Illustrated)  (Spanish)
- **Goal:** Infection prevention information better understood(35)(36)(37)
 - **Interventions:**
 - **ASSIST:** Identifying patient and caregiver's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling to evaluate infection prevention problems, if needed
 - **ASSIST:** Identifying exposure risks and ways to reduce risk
 - **ASSIST:** Identifying vaccination needs and considerations
 - **EDUCATE:** Vaccination precautions when immunocompromised(34):

- Avoiding live vaccines (eg, live vaccines for measles, mumps, or chicken pox)
- Direct contact with people who have just been vaccinated is allowed (except smallpox).
- Discuss with provider about direct contact after a caregiver or family member gets a vaccine.
- **EDUCATE:** Adhere to strict infection control procedures due to effects of immunosuppression or corticosteroids.
- **EDUCATE:** Infection prevention techniques; things to do:
 - Practice good hand-washing techniques, and wash hands frequently. Use alcohol-based hand sanitizer if soap and water are not available.
 - Inspect skin daily for lesions or breaks, and protect from cuts and burns.
 - Perform mouth care before and after meals, or 3 to 4 times daily.
 - Perform perineal care after voiding and bowel movements.
 - Wear gloves in situations with high risk of infection (eg, gardening, handling pet feces).
 - Use latex condoms during any sexual contact.
 - Importance of immunizations to reduce risk of infection (for patient and caregivers)
 - Antiviral prophylaxis may be needed for the prevention of cytomegalovirus (CMV) replication and disease after solid organ transplant.(20)(38)
- **EDUCATE:** Infection prevention techniques; things to avoid:
 - Sharing personal hygiene items (eg, razors, bath towels) and food utensils
 - Crowded places
 - People with infections (especially colds)
 - Live vaccines (eg, oral vaccines for polio, varicella, and smallpox, or nasal flu vaccine)(34)
 - Caregivers or family who have a rash after a varicella vaccine; do not have direct contact until the rash goes away.(34)
 - Stagnant water (eg, in vases), where bacterial growth may occur
 - Cold-mist humidifiers, which can harbor bacteria
 - Exotic animals and pet birds
 - Construction or renovation areas (eg, carpet removal) and air vent cleaning
 - Working in healthcare facilities, correctional institutions, shelters for the homeless, or any other designated high-risk location
- **EDUCATE:** Proper food handling to reduce risk of infection(39)(40)(41):
 - Wash hands after contact with raw meat.
 - Ensure that all meat, poultry, and fish is thoroughly cooked.[D]
 - Do not eat raw or undercooked eggs (which may be found in hollandaise sauce, Caesar salad dressings, mayonnaise, uncooked cookie dough, or raw batter).
 - Cook leftover food until steamy hot.
 - Wash all fruits and vegetables well.
 - Do not drink unpasteurized fruit juices.
 - Do not eat vegetable seed sprouts of any kind.
 - For foods such as salsas or salad dressings, use only shelf-stable foods (that is, unopened canned, bottled, or packaged food that can be stored at room temperature until opened).
 - Avoid soft cheeses and unpasteurized milk and cheese products.
 - Prepared meats (eg, hot dogs, luncheon meats, or cold cuts) must be heated to steaming (internal temperature of 165 degrees F (73.9 degrees C)).(42)
 - Eggs must be cooked until the yolks and whites are firm (internal temperature of 160 degrees F (71.2 degrees C)).(42)
 - Avoid cream-filled sweets.
 - Do not eat raw honey.
 - Casseroles must attain an internal temperature of 165 degrees F (73.9 degrees C).(42)
- **EDUCATE:** Dining out precautions:
 - Go to places that are smaller or go at times when no crowds will be there.
 - Ask if food is prepared fresh rather than just warmed up.
 - Ask if juices are pasteurized.
 - Avoid places where foods are touched a lot or may not be stored carefully (buffets, delicatessens, prepared sandwiches, potlucks, salad bars, and sidewalk vendors).
- **EDUCATE:** Proper water precautions to reduce risk of infection:
 - Tap water is acceptable to drink in most cities or municipalities.
 - Most nationally distributed bottled water products and fruit juices are considered safe.
 - Use filtered water (if applicable); portable water filters and refrigerated water dispensers do not meet standards for safety.
 - Description on filter must ensure removal of all coliforms and cryptosporidia.
 - Bottled water must be labeled as having been treated with reverse osmosis, distillation, and filter of 1 micron or smaller.
- **EDUCATE:** Travel precautions:

- Avoid traveling to developing countries.
- If traveling to developing countries, avoid street vendors, raw food, and ice cubes; boil all water for at least 1 minute or use only commercially bottled water; and carry antibiotic prophylaxis or antimicrobial agents, if recommended by provider.
- **COORDINATE:** Vaccinations
- **COORDINATE:** Home care referral, if indicated(10)
- **COORDINATE:** Social worker referral, as needed(8)
- **COORDINATE:** Care team meeting, if needed(12)(13)
- **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
- **COORDINATE:** Follow-up contact to evaluate infection prevention understanding, identification of problems, and need for further assistance(16)(17)
- **SEND:** Vaccines after your transplant  (Spanish)
- **SEND:** Coronavirus disease 2019 (COVID-19) vaccines  (Spanish)
- **SEND:** Flu vaccine  (Spanish)
- **SEND:** Hepatitis A vaccine  (Spanish)
- **SEND:** Hepatitis B vaccine  (Spanish)
- **SEND:** Food safety tips  (Spanish)
- **SEND:** How to wash your hands (Illustrated)  (Spanish)
- **SEND:** Managing your medicines (Illustrated)  (Spanish)
- **SEND:** Pneumonia vaccine for adults  (Spanish)
- **SEND:** Pneumonia vaccine for children  (Spanish)
- **SEND:** Prevent infections  (Spanish)
- **SEND:** Stay away from germs - Immune problems  (Spanish)
- **SEND:** Important phone numbers when you travel  (Spanish)
- **SEND:** Before you travel  (Spanish)
- **SEND:** Travel tips - Immune problems  (Spanish)
- **SEND:** Water precautions - Immune problems  (Spanish)
- **SEND:** When to get help - Organ transplant  (Spanish)
- **Goal:** Nutritional needs met and nutritional plan understood(43)(44)(45)(46)
 - **Interventions:**
 - **ASSIST:** Identifying patient and caregiver's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for evaluation and assessment of nutritional needs
 - **EDUCATE:** Nutritional plan may include:
 - Calories based on dietary reference intake (DRI) and transplant type
 - Dietary restrictions of simple sugars
 - Fiber increase to prevent constipation
 - Importance of food safety
 - Moderate-fat diet with healthy fats: olive oil, fish, and nuts
 - Adequate protein and calories to promote healing
 - Sodium restriction possible with hypertension
 - Value of nutritional assessment
 - Avoid grapefruit products. Grapefruit juice can alter the effects of immunosuppressive drugs, boosting them to toxic levels.(47)
 - Avoid high-fat entrees when eating out.
 - Close monitoring of growth and development in pediatric patients(48)
 - Need for enteral nutrition, if indicated
 - Take multivitamin-mineral supplement as prescribed (eg, anemia, vitamin D deficiency).
 - Weight loss, if indicated
 - **COORDINATE:** Dietitian referral, if indicated
 - **COORDINATE:** Endocrinologist referral, if indicated
 - **COORDINATE:** Gastroenterologist referral, if indicated
 - **COORDINATE:** Rehabilitation therapy referral, if indicated[C]
 - **COORDINATE:** Laboratory testing, if indicated

- **COORDINATE:** Follow-up contact with patient to evaluate understanding and adherence to self-care plan, referral status, and need for further assistance(16)(17)
 - **SEND:** DASH eating plan  (Spanish)
 - **SEND:** Food safety tips  (Spanish)
 - **SEND:** Water precautions - Immune problems  (Spanish)
 - **SEND:** Travel tips - Immune problems  (Spanish)
 - **SEND:** Eating to keep up your energy  (Spanish)
 - **SEND:** Tips for getting your child to eat more  (Spanish)
- **Goal:** Preconception counseling or pregnancy management implemented and maintained
 - **Interventions:**
 - **ASSIST:** Identifying patient and caregiver's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for discussion about contraception
 - **ASSIST:** Appointment scheduling with transplant-specialized obstetrician, if considering pregnancy
 - **EDUCATE:** Contraceptive counseling for transplant recipients may include(18)(49):
 - Delay pregnancy for 1 to 2 years depending on transplant team recommendations and organ replaced.(50)
 - Types and efficacy of contraceptive methods. Most types of contraceptives are acceptable in patients with uncomplicated transplants. Comorbidities, drug interactions, and post-transplant complications may limit the use of oral contraceptives.(51)(52)
 - Correct use based on contraceptive (eg, proper application of condom, placement of diaphragm, administration timing of hormonal contraceptive medication)
 - Contraceptive risks (eg, thromboembolism risk with hormonal contraceptives, urinary tract infection risk with spermicides, pelvic inflammatory disease with IUD)
 - Impact of medications (eg, antibiotics) on effectiveness of hormonal contraception
 - **EDUCATE:** Notify transplant center immediately if pregnant.
 - **EDUCATE:** Manage as high-risk pregnancy when post transplant.
 - **COORDINATE:** Preconception specialist referral, as needed
 - **COORDINATE:** Obstetrician referral, if indicated
 - **COORDINATE:** Care team meeting, if needed(12)(13)
 - **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
 - **COORDINATE:** Follow-up contact with patient to evaluate understanding and adherence to preconception self-care plan, referral status, and need for further assistance(16)(17)
- **Goal:** Pediatric patient transition planning to adult providers or transplant services initiated(53)(54)
 - **Interventions:**
 - **ASSIST:** Identifying patient and caregiver's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Creation of transition plan to adult providers
 - **ASSIST:** Assessing child's readiness for transition
 - **ASSIST:** Appointment scheduling and preparation with adult providers of care
 - **EDUCATE:** Transition plan to adult care providers:
 - Deliver developmentally appropriate care that is well timed, coordinated, and provides uninterrupted treatment.
 - Comprehensive to meet the needs of adolescents and young adults
 - Importance of sharing information about the transition process
 - Ensures the young adult has the necessary independence, self-care, and decision-making skills
 - **COORDINATE:** Transition to adult provider, as needed
 - **COORDINATE:** Care team meeting, if needed(12)(13)
 - **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
 - **COORDINATE:** Follow-up contact with patient to discuss transition planning outcomes, referral status, and need for further assistance(16)(17)
 - **SEND:** Be ready for your doctor visit - Organ transplant  (Spanish)
 - **SEND:** Be ready with questions for your doctor - Organ transplant  (Spanish)

P4. Transplant medical home needed

- **Goal:** Medical home plan developed, implemented, and maintained(55)(56)
 - **Interventions:**

- **ASSIST:** Identifying patient and caregiver's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
- **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
- **ASSIST:** Finding and facilitating patient's care through a medical home(57)
- **ASSIST:** Appointment scheduling
- **ASSIST:** Communication between multiple providers(11)
- **EDUCATE:** Medical home purpose
- **EDUCATE:** Role and participation in the medical home
- **EDUCATE:** Use and participation in medical home telehealth (eg, electronic medical records and patient portals, virtual house calls, video consulting, and medical devices such as digital stethoscope or high-resolution examination cameras) (58)
- **COORDINATE:** Access to care
- **COORDINATE:** Transitions between care settings
- **COORDINATE:** Care team meeting, if needed(12)(13)
- **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
- **COORDINATE:** Follow-up contact to review medical home plan understanding, referral status, and need for further assistance(16)(17)

P5. Transplant potential complications and comorbidities monitoring needed

- **Goal:** Transplant potential complication or comorbidity evaluated and addressed(4)(5)[N](#)
 - **Interventions:**
 - **ASSIST:** Identifying patient and caregiver's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Identification of comorbidities and their treatments that may make condition management more difficult
 - **ASSIST:** Appointment scheduling for evaluation of problems, comorbidities, and self-care plan[E]:
 - Appointment scheduling with provider for evaluation of alternative treatment options (eg, medication changes, invasive treatment, palliative care)
 - Recently hospitalized patient's evaluation within 1 week of discharge[N](#)
 - Recently hospitalized patient's evaluation with outpatient healthcare providers (eg, nurse, social worker) within 1 to 3 days of discharge(16)
 - Follow-up appointment as indicated by transplant team
 - **EDUCATE:** When and how to get help for worsening transplant problems
 - **EDUCATE:** Treatment and monitoring for comorbid conditions (eg, coronary artery disease, high blood pressure, heart valve disease)(11)
 - **EDUCATE:** Encourage routine ophthalmologic, dermatologic, and dental examinations.
 - **EDUCATE:** Follow-up appointments for assessment of clinical status
 - **EDUCATE:** Importance of monitoring for common complications, such as(20):
 - Anemia
 - Bone loss and fractures
 - Cancer
 - Cardiovascular disease
 - Depression
 - Gout
 - Graft vs host disease(21)
 - Growth retardation(59)
 - Hypertension
 - Infections
 - Insomnia
 - Medication side effects
 - Metabolic syndrome or new-onset diabetes
 - Peptic ulcer disease: upper abdominal pain, hematemesis, and melena(60)
 - Peripheral neuropathy(19)
 - Pneumonia
 - Post-transplant lymphoproliferative disorder: diarrhea, lower GI bleeding, severe abdominal pain, mononucleosis-like symptoms (eg, fever, enlarged lymph nodes, fatigue), headache or confusion, and new mass lesions(21)
 - Sarcopenia[F](61)
 - Sexual problems
 - Weight gain
 - **COORDINATE:** Specialist referral, if indicated
 - **COORDINATE:** Laboratory testing, if indicated

- **COORDINATE:** Outpatient imaging, tests, or procedures, if indicated
- **COORDINATE:** Care team meeting, if needed(12)(13)
- **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
- **COORDINATE:** Follow-up contact to review self-care plan and need for further assistance(16)(17)
- **SEND:** Be ready for your doctor visit - Organ transplant  (Spanish)
- **SEND:** Be ready with questions for your doctor - Organ transplant  (Spanish)
- **SEND:** Be ready for your doctor visit - Osteoporosis  (Spanish)
- **SEND:** Be ready with questions for your doctor - Osteoporosis  (Spanish)
- **SEND:** Healthy bones - Calcium and vitamin D  (Spanish)
- **SEND:** How to check your heart rate during exercise (Illustrated)  (Spanish)
- **SEND:** How to measure your heart rate (Illustrated)  (Spanish)
- **SEND:** Osteoporosis - Medicines  (Spanish)
- **SEND:** Exercising when you have osteoporosis  (Spanish)
- **SEND:** Mouth sores or infections  (Spanish)

P6. Transplant medication evaluation needed

- **Goal:** Transplant medication evaluation completed and medication regimen initiated, if indicated
 - **Interventions:**
 - **ASSIST:** Identifying patient and caregiver's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with prescribing provider for evaluation of condition status
 - **EDUCATE:** When and how to get help for worsening organ transplant problems
 - **EDUCATE:** Medication management needs common for organ transplant
 - **COORDINATE:** Follow-up after provider contact for evaluation of medication changes(16)(17)

P7. Transplant medication management needed

- **Goal:** Medication information understood(62)
 - **Interventions:**
 - **ASSIST:** Identifying patient and caregiver's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for discussion on appropriate medication
 - **ASSIST:** Providing reconciled medication list to all providers. See Medication Reconciliation Tool  SR for more information.
 - **EDUCATE: Purpose:** Importance of understanding what each medication is for, how it should be taken, and what to do if dose is missed
 - **EDUCATE:** Medication indications, side effects, and considerations frequently change. See a drug reference for specific information not listed here.
 - **EDUCATE:** Administration techniques:
 - Route (eg, oral, intramuscular (IM), intravenous (IV))
 - With or without foods or liquids (eg, milk)
 - Timing of administration
 - Appropriate measuring device (eg, liquid medications)
 - Proper storage
 - **EDUCATE: Get emergency help:** Adverse or allergic reactions (eg, wheezing, chest tightness, fever, itching, bad cough, blue skin color, hemorrhage, seizures, swelling of face, lips, tongue, or throat)
 - **EDUCATE: Side effects:** Management, and when to contact provider
 - Common side effects include gastrointestinal changes, dry mouth, fatigue, insomnia, rash, dizziness, and headache.
 - **EDUCATE: Considerations:**
 - Discuss medication with provider if pregnant, planning a pregnancy, or breast-feeding.
 - Comorbidities may be contraindications to the use of a drug (eg, liver disease, diabetes).
 - Health checks, diary, and action plans (eg, blood pressure, weight, heart rate)
 - Safety measures (eg, sun protection, driving, or operating machinery)
 - Contact provider or pharmacist prior to taking any other over-the-counter or prescribed medication.
 - **COORDINATE:** Clinical monitoring appropriate to medication use (eg, laboratory tests)

- **COORDINATE:** Communication between patient and pharmacy (for obtaining prescriptions, setting up home medication delivery, review for drug-drug interactions)^(N)
- **COORDINATE:** Communication between care team members and medication list reconciliation
- **COORDINATE:** Follow-up to evaluate adherence or problems with medication, provide additional education and support, and identify need for further assistance⁽¹⁶⁾⁽¹⁷⁾
- **COORDINATE:** Laboratory testing, if indicated
- **COORDINATE:** Outpatient imaging, tests, or procedures, if indicated
- **COORDINATE:** Therapeutic drug level monitoring
- **SEND:** Managing your medicines (Illustrated)  (Spanish)

P8. Corticosteroid medication management needed

- **Goal:** Corticosteroid initiated and maintained⁽⁴⁸⁾⁽⁶³⁾
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Corticosteroids block the normal effects of the immune system to prevent transplant rejection.
 - **EDUCATE: Call provider immediately:** Edema, hypertension, vision changes, bone fracture, severe mood changes
 - **EDUCATE: Side effects:** Increased appetite, fluid retention, weight gain, mood alteration, sleep disturbance, and peptic ulcer
 - Long-term use associated with adrenal axis and growth suppression, hypertension, diabetes, Cushing syndrome, and dermatologic (eg, skin thinning, striae, acne), ophthalmic (eg, glaucoma, cataracts), and musculoskeletal (eg, necrosis, osteoporosis) effects
 - **EDUCATE: Considerations:**
 - Taper oral course when stopping.
 - Steroid dependency possible
 - Avoid live virus immunizations.
 - Bone density loss management

P9. Immunosuppressive medication management needed

- **Goal:** Immunosuppressive medication initiated and maintained^{[G](47)(63)(64)}
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Immunosuppressive medication blocks the normal effects of the immune system to prevent transplant rejection.
 - Azathioprine, basiliximab, belatacept, and sirolimus block the normal effects of the immune system to prevent kidney transplant rejection.
 - Mycophenolate and cyclosporine block the normal effects of the immune system to prevent transplant rejection.
 - **EDUCATE: Call provider immediately:** Hepatotoxicity (eg, clay-colored stools, dark urine, pruritus, jaundice), infection signs (eg, fever, sore throat), neurologic symptoms, unusual bleeding or bruising
 - Toxicity signs (eg, nausea, vomiting, diarrhea, abnormal liver function, leukopenia)
 - **EDUCATE: Side effects:** Cold hands and feet, hair thinning, leukopenia, joint pain, anorexia, tremors, hypertension, and GI upset
 - Increased risk for infection and malignancy
 - **EDUCATE: Considerations:**
 - Dosage based on weight and laboratory testing
 - Hold medication on day of blood draw.
 - Initiate prophylactic antibiotic regimen at first onset of symptoms or as prescribed (eg, prior to dental or surgical procedures).
 - Infection prevention measures (eg, hand hygiene, avoid crowds)
 - Skin care management (eg, hygiene, wound prevention)
 - Oral care and dental examination regularly, as drug may cause gum disease
 - Avoid in patients with liver or kidney dysfunction.
 - **EDUCATE:** Avoid while taking immunosuppressives:
 - Live virus immunizations
 - People who have recently gotten live virus immunizations
 - Ill people, even those with colds
 - Contact sports
 - Diet considerations with cyclosporine (eg, avoid foods high in potassium (eg, whole grains, legumes, meat, bananas, apricots, orange juice, potatoes, and raisins), eating grapefruit, or drinking grapefruit juice)⁽⁶⁵⁾
 - **COORDINATE:** Dentist referral, if indicated
 - **SEND:** Immunosuppressant medicines  (Spanish)

P10. Respiratory problem evaluation needed

- **Goal:** Respiratory problems evaluated and managed
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for respiratory problem evaluation and creation of self-care plan
 - **EDUCATE:** How to monitor for problems and when and how to get help
 - **COORDINATE:** Follow-up to evaluate respiratory status, referral status, and need for further assistance(16)(17)

P11. Respiratory complications information needed

- **Goal:** Respiratory complications better understood
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for respiratory problem evaluation and creation of self-care plan
 - **ASSIST:** Self-care plan creation between patient and provider (eg, goals, management, treating symptoms, getting help)
 - **ASSIST:** Communication between multiple providers(25)
 - **EDUCATE:** Respiratory self-care program(66):
 - Importance of treatments
 - Medication role
 - Complication prevention (stay inside on "code red" days, avoid triggers)(67)
 - Equipment use and care
 - **EDUCATE:** Airway discomfort care and oxygenation promotion(68):
 - Sit with elbows resting on knees.
 - Lean forward over back of chair or against wall.
 - Elevate head of bed 30 degrees (or support head with 2 pillows when in bed).
 - When in bed, turn at least every 2 hours to promote drainage of secretions.
 - Breathing exercises (eg, controlled cough, pursed-lip breathing)
 - **EDUCATE:** Factors associated with increased risk of respiratory problems (eg, smoking, chemotherapy)
 - **EDUCATE: When to get emergency help(69)(70):**
 - Respiratory impairment (eg, gasping for air, choking, cyanosis, chest pain, severe dyspnea)(69)
 - Heartbeat irregular, new
 - **EDUCATE: When to call the provider(71):**
 - Cold or flu symptoms that do not go away
 - Cough that does not stop
 - Change in color of phlegm (mucus)
 - Dyspnea makes it harder to say a sentence, or cannot walk across the room.
 - Breathing medication problems (eg, increased use, decreased effectiveness)
 - Pain when breathing
 - Trouble eating or sleeping because short of breath
 - Medication problems (eg, side effects, access, administration)
 - Medical equipment problems or needs
 - **COORDINATE:** Pulmonologist referral, if indicated
 - **COORDINATE:** Medical equipment and supplies, if indicated
 - **COORDINATE:** Home care referral, if indicated(68)
 - **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
 - **COORDINATE:** Follow-up to evaluate respiratory status, referral status, and need for further assistance(16)(17)

P12. Urinary tract infection symptoms present

- **Goal:** Urinary tract infection symptoms improved or resolved
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for urinary tract infection evaluation
 - **EDUCATE:** Medication side effects that may cause urinary retention (and increase risk of urinary tract infection)
 - **EDUCATE:** Urinary tract infection prevention measures(72)(73):
 - Proper external skin hygiene

- Avoid bladder irritants (eg, alcohol, caffeine, and tobacco).(74)
- Empty bladder regularly.
- **COORDINATE:** Urologist referral, if indicated
- **COORDINATE:** Laboratory testing, if indicated
- **COORDINATE:** Outpatient imaging, tests, or procedures, if indicated
- **COORDINATE:** Home care referral, if indicated
- **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
- **COORDINATE:** Follow-up contact to evaluate urinary tract status, referral status, and need for further assistance(16)(17)
- **SEND:** Bladder care  (Spanish)

P13. Urinary retention information needed

- **Goal:** Urinary retention managed
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Self-care plan creation between patient and provider (eg, goals, management, treating symptoms, getting help)
 - **EDUCATE:** Urine retention management(74)(75):
 - Drink coffee or caffeinated tea to create urge to void.
 - Do not drink alcohol (it leads to urinating a lot and a risk of not voiding until bladder is distended due to less sensation).
 - Apply a warm compress to the lower abdomen.
 - Sit in tub of warm water or take warm shower.
 - Drink small amounts of fluid throughout the day (not large amounts at once).
 - Keep bladder from becoming overdistended (generally less than 400 mL).
 - Bladder program techniques
 - Urinary catheter use and care, if appropriate
 - If these techniques do not work, seek care immediately.
 - **EDUCATE:** Medication side effects that may cause urinary retention
 - **COORDINATE:** Urologist referral, if indicated
 - **COORDINATE:** Laboratory testing, if indicated
 - **COORDINATE:** Outpatient imaging, tests, or procedures, if indicated
 - **COORDINATE:** Home care referral, if indicated
 - **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
 - **COORDINATE:** Follow-up contact to evaluate urinary retention, referral status, and need for further assistance(16)(17)

P14. Bladder program information needed

- **Goal:** Bladder program understood and followed(76)(77)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Self-care plan creation between patient and provider (eg, goals, management, treating symptoms, getting help)
 - **EDUCATE:** Bladder program(73)(78):
 - Keep voiding diary.
 - Void on schedule or with urge.
 - Practice double-voiding (try to void, then wait 2 minutes and try again).
 - Increase time between voids (eg, 30 minutes a week, as tolerated).N
 - Keep bladder from becoming overdistended (generally less than 400 mL).
 - Maintain steady fluid intake (within dietary limits).
 - Avoid bladder irritants (eg, alcohol, caffeine, and tobacco).(76)
 - When and how to get help for problems
 - Maintain a healthy weight.
 - Manage constipation.
 - **COORDINATE:** Medical equipment and supplies, if indicated
 - **COORDINATE:** Home care referral, if indicated
 - **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
 - **COORDINATE:** Follow-up contact to evaluate bladder program, referral status, and need for further assistance(16)(17)

P15. Urinary incontinence management needed

- **Goal:** Urinary incontinence managed(76)(79)

- o **Interventions:**

- **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
- **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
- **ASSIST:** Appointment scheduling with provider for urinary problem evaluation and creation of self-care plan
- **ASSIST:** Self-care plan creation between patient and provider (eg, goals, management, treating symptoms, getting help)
- **EDUCATE:** Urine incontinence management(74)(78):
 - Bladder program techniques
 - Keep pathway to bathroom clear and bathroom clutter-free.
 - Take regular bathroom breaks; use toilet before leaving house.
 - Wear clothes that are easy to get on and off.
 - Ways to suppress urge (eg, counting or pelvic muscle contraction)
 - Avoid drinking fluids 2 hours before bedtime to keep from waking up a lot to urinate.
 - Avoid bladder irritants (eg, alcohol, caffeine, and tobacco).
 - Avoid taking diuretic medication after 4 pm.
 - Bladder expression using Valsalva or Crede maneuvers, if appropriate
 - Tobacco cessation plan
 - Urinary catheter use and care, if appropriate
 - Medications for bladder problems: urologic anticholinergic and cholinergic drugs, if ordered
 - Weight loss techniques, if appropriate^[H](80)
- **COORDINATE:** Urologist referral, if indicated(80)
- **COORDINATE:** Tobacco use treatment, as needed
- **COORDINATE:** Laboratory testing, if indicated
- **COORDINATE:** Outpatient imaging, tests, or procedures, if indicated
- **COORDINATE:** Home care referral, if indicated
- **COORDINATE:** Medical equipment and supplies, if indicated
- **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
- **COORDINATE:** Follow-up contact to evaluate urinary incontinence, referral status, and need for further assistance(16)(17)
- **SEND:** Bladder care  (Spanish)
- **SEND:** How to stop smoking  (Spanish)

P16. Information needed on when to get help with urinary problem

- **Goal:** Urinary problems evaluated by provider when needed
 - o **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for urinary problem evaluation and creation of self-care plan
 - **EDUCATE:** When to get emergency help(72)(81):
 - Urine pattern change, sudden (eg, not making urine)
 - Abdominal pain, severe, unrelieved by analgesics
 - Bladder overdistended (eg, pounding headache, sweating for no reason, cold hands or feet, feeling anxious)
 - **EDUCATE:** When to call the provider(72)(81):
 - Bowel or bladder dysfunction
 - Urinary problems (eg, pain, frequency change, blood)(74)
 - Urine problems (eg, cloudy, dark yellow, or brown)
 - Urine stones
 - **EDUCATE:** Medication management needs common for patient's condition
 - **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
 - **COORDINATE:** Follow-up contact to evaluate patient status, referral status, and need for further assistance(16)(17)

P17. Blood pressure monitoring information needed

- **Goal:** Blood pressure self-monitoring improved
 - o **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)

- **ASSIST:** Appointment scheduling with provider for evaluation of blood pressure problems or creation of self-care plan to include:
 - Obtaining home blood pressure monitor with correct cuff size for patient
 - Identifying blood pressure targets
- **EDUCATE:** Why blood pressure monitoring is important
- **EDUCATE:** When to check blood pressure
- **EDUCATE:** How to check blood pressure(82)(83)(84):
 - Take blood pressure medication at same time each day.
 - Avoid smoking, caffeine intake, and exercise for 30 minutes prior to taking blood pressure.
 - Fit and position cuff properly (cuff should be appropriate size for patient). Do not put cuff over clothing.
 - Support arm on flat surface at level of heart, with back supported and both feet on floor, for at least 5 minutes prior to taking blood pressure.
 - Measure blood pressure in same arm each reading.[I](82)
 - Blood pressure goal and the need to know own blood pressure from recent measurement
 - Keeping a diary of blood pressure readings and reviewing diary with provider at appointments
- **EDUCATE: When to call the provider:**
 - Blood pressure outside of provider's parameters
 - There is a 10 mm Hg or greater difference in pressure between right and left arm.[J](85)
- **COORDINATE:** Home care referral, if indicated
- **COORDINATE:** Follow-up contact to evaluate blood pressure monitoring, referral status, and need for further assistance(16)(17)
- **SEND:** How to check your blood pressure (Illustrated)  (Spanish)
- **SEND:** Blood pressure diary (Illustrated)  (Spanish)

P18. Fall risk evaluation needed

- **Goal:** Falls prevented and fall management plan followed
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling for medical evaluation of functional changes and creation of self-care plan
 - **ASSIST:** Creation of rehabilitation treatment plan between patient and care team (therapists and other providers) 
 - **EDUCATE:** Fall risk identification(86)(87):
 - Age
 - Female
 - Low body mass
 - Behavioral health comorbidities
 - Musculoskeletal diseases
 - Cognitive impairment
 - Gait and balance problems
 - History of falls
 - Improper footwear (eg, backless shoes or slippers, shoes without nonskid soles, high-heeled shoes, or smooth leather soles)
 - Sensory impairments
 - Orthostatic hypotension episodes
 - Medications: antiarrhythmics, antidepressants, antihypertensives, antiseizure medications, benzodiazepines, and diuretics(88)(89)(90)
 - Pain
 - Medical comorbidities
 - **EDUCATE:** Home changes to help prevent falls:
 - Remove all throw rugs and clutter from floors.
 - Repair carpet tears, broken tiles, or hardwood floor ruts that could cause tripping.
 - Clear walking areas of electrical cords or wires.
 - Move furniture to allow for use of crutches or walker.
 - Keep all areas well lit.(91)
 - Clean up spills immediately.
 - **EDUCATE:** Self-care safety activities(92)(93):
 - Rise slowly from lying or sitting position.
 - Wear glasses or contact lenses, if needed.
 - Use handrails, grab bars, or adaptive devices as needed.
 - Wear shoes with nonslip soles.

- Use no-slip mats in shower or tub.
- Pet-related falls mitigation (eg, not chasing pet, obedience training)
- Know which medication may cause problems (eg, dizziness, weakness, or balance problems).(94)
- Exercise program within provider's recommendations(26)(95)[N](#)
- **EDUCATE: When to get emergency help:**
 - Fall complications (eg, head injury, broken bone, significant ADL changes)
 - Fear of personal harm from someone (eg, spouse, parent).(96) See Intimate Partner Violence [CCG](#).
 - Stroke, heart attack, or blood clot
- **EDUCATE: When to call the provider:**
 - Cognitive changes (eg, thinking problems, forgetfulness worsens)
 - Physical activity level declines (eg, fatigue, weakness).
 - Depression (eg, feeling sad, hopeless, overwhelmed)(33)(97)
 - Medication problems (eg, side effects, access, administration)
 - Self-care changes (eg, ADL or IADL problems)
 - Falls
- **COORDINATE:** Pharmacy medication support, if indicated[K](86)
- **COORDINATE:** Neurologist referral, if indicated
- **COORDINATE:** Rehabilitation therapy referral, if indicated(98)(99)
- **COORDINATE:** Abuse or violence advocacy, as needed
- **COORDINATE:** Medical equipment and supplies, if indicated(100)(101)
- **COORDINATE:** Community services referral, as needed[L](102)
- **COORDINATE:** Social worker referral, as needed(8)
- **COORDINATE:** Home care referral, if indicated(99)(103)
- **COORDINATE:** Outpatient imaging, tests, or procedures, if indicated(104)
- **COORDINATE:** Care team meeting, if needed(12)(13)
- **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
- **COORDINATE:** Follow-up contact with patient to evaluate fall risk, referral status, and need for further assistance(16)(17)
- **SEND:** Choosing a place to live [Illustrated](#) (Spanish)
- **SEND:** Exercising to improve balance (Illustrated) [Illustrated](#) (Spanish)
- **SEND:** Exercising when you have neurologic problems [Illustrated](#) (Spanish)
- **SEND:** How to get out of bed safely (Illustrated) [Illustrated](#) (Spanish)
- **SEND:** How to use a walker (Illustrated) [Illustrated](#) (Spanish)
- **SEND:** Safety in the home - Fall protection [Illustrated](#) (Spanish)
- **SEND:** Ways to save energy [Illustrated](#) (Spanish)
- **SEND:** Get ready to use a wheelchair at home [Illustrated](#) (Spanish)
- **SEND:** How to use a cane (Illustrated) [Illustrated](#) (Spanish)

P19. Physical function impairments present

- **Goal:** Physical functioning improved
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for medical evaluation of functional changes and creation of self-care plan
 - **ASSIST:** Creation of rehabilitation treatment plan between patient and care team (therapists and other providers)(105)
 - **ASSIST:** Support system (eg, family, friends) identification, enlistment, and education
 - **EDUCATE:** Ways to manage physical impairments with self-care plan(100):
 - Importance of physical activity to maintain full functional level
 - Value of rehabilitation(98)
 - Use of assistive devices
 - Exercise recommendations
 - Home environmental safety
 - Transportation safety: modified van with lift, sensitized hand controls, hand controls
 - **EDUCATE:** When and how to get help for problems:
 - Fear of personal harm from someone (eg, spouse, parent).(96) See Intimate Partner Violence [CCG](#).

- When to get emergency help because of ADL problems (eg, fall with loss of consciousness, stroke or heart attack signs, lack of oxygen)
- When to call the provider for ADL problems (eg, needing more help to perform, change of condition)
- **COORDINATE:** Neurologist referral, if indicated
- **COORDINATE:** Rehabilitation therapy referral, if indicated(105)
- **COORDINATE:** Medical equipment and supplies, if indicated(100)
- **COORDINATE:** Community services referral, as needed(102)
- **COORDINATE:** Social worker referral, as needed(8)
- **COORDINATE:** Home care referral, if indicated(103)
- **COORDINATE:** Care team meeting, if needed(12)(13)
- **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
- **COORDINATE:** Follow-up contact with patient to evaluate physical impairments, referral status, and need for further assistance(16)(17)
- **SEND:** Choosing a place to live  (Spanish)
- **SEND:** How to move safely in bed (Illustrated)  (Spanish)
- **SEND:** Exercising to improve balance (Illustrated)  (Spanish)
- **SEND:** Exercising when you have neurologic problems  (Spanish)
- **SEND:** Safety in the home - Fall protection  (Spanish)
- **SEND:** Get ready to use a wheelchair at home  (Spanish)
- **SEND:** Ways to save energy  (Spanish)

P20. Cognitive impairment present

- **Goal:** Cognitive and behavioral symptoms addressed and managed(106)(107)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Support system (eg, family, friends) identification, enlistment, and education
 - **ASSIST:** Appointment scheduling for evaluation of new or worsening change in behavioral or cognitive function, and the development of a self-care plan
 - **ASSIST:** Creation of a self-care plan between patient and provider (eg, goals, management, treating symptoms, getting help)
 - **ASSIST:** Communication between multiple providers(25)
 - **EDUCATE:** Cognition information(108):
 - Cognition is ability to learn, solve problems, remember, and appropriately use information.
 - Problems or deficits are part of aging process, but specific disorders may be treatable (eg, delirium, dementia, depression).
 - Cognition deficits may be progressive.
 - When and how to get help if cognition problems progress to point caregiver cannot provide care
 - **EDUCATE:** Ways to help manage cognitive changes(108)(109):
 - Agitation management (eg, simplify tasks and instructions, keep routine consistent)
 - Maintain a calm environment.
 - Set small goals every day (eg, get out of bed at a certain time, finish one chore or project, or do something that brings pleasure, such as listening to music).
 - Structured activities (eg, exercise therapy, pet therapy, social contact)
 - Focus on process instead of results.
 - Break down all tasks into simple steps.
 - ADL/IADL scheduled at times during the day when better able to perform or participate
 - Encourage participation in ADL and IADL, but do not force.
 - Organizational strategies and problem-solving tasks, including note-taking devices, cueing, and using memory books
 - Mentally stimulating activities participation (eg, reading, puzzles)
 - Orientation aids (eg, calendars, clocks, and alarms)
 - **EDUCATE:** Medication side effects for cognitive and behavioral deficits (eg, antipsychotics, tricyclic antidepressants, SSRI antidepressants, dementia drugs)
 - **EDUCATE:** Cognitive deficits may be progressive, and how to get help if they progress to point caregiver cannot provide care.
 - **EDUCATE: When to get emergency help:**
 - Suicidal or homicidal actions or plans(97)

- Psychotic behavior (eg, threatening)
- Heart attack (eg, persistent chest pain; pain radiating to the arms, shoulders, neck, back, and/or jaw; cool clammy skin)(28)
- Stroke (eg, face drooping, arm weakness, speech problems, bad headache, vision changes, loss of balance or coordination)(29)
- **EDUCATE: When to call the provider**(109):
 - Depression (eg, feeling sad, hopeless, overwhelmed)(97)
 - Hallucinations or delusions increase.
 - Cognitive changes (eg, thinking problems, forgetfulness worsens)
 - Insomnia
 - Self-care changes (eg, ADL or IADL problems)
- **COORDINATE:** Behavioral health services referral, if indicated
- **COORDINATE:** Home care referral, if indicated
- **COORDINATE:** Neurologist referral, if indicated
- **COORDINATE:** Psychiatrist referral, if indicated
- **COORDINATE:** Community services referral, as needed
- **COORDINATE:** Social worker referral, as needed(8)
- **COORDINATE:** Support group referral, as needed
- **COORDINATE:** Rehabilitation therapy referral, if indicated[M](107)
- **COORDINATE:** Caregiver health service referral, as needed
- **COORDINATE:** Care team meeting, if needed(12)(13)
- **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
- **COORDINATE:** Follow-up evaluation for problems due to cognitive impairment, referral status, and need for further assistance(16)(17)
- **SEND:** Caregiver tips  (Spanish)
- **SEND:** Helping people who cannot think clearly  (Spanish)
- **SEND:** Safety in the home - Memory loss  (Spanish)
- **SEND:** Help with sleeping or insomnia  (Spanish)
- **SEND:** Tips for good mental health  (Spanish)
- **SEND:** Recognizing the signs of depression  (Spanish)

P21. Weight change monitoring information needed

- **Goal:** Weight monitoring understood
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for evaluation of weight changes and creation of self-care plan to include:
 - Establishing target goals for weight gain or loss based on patient's clinical needs
 - Creation of weight diary to aid in monitoring
 - **EDUCATE:** Daily weight monitoring includes(110):
 - Establish "dry weight" based on home scale readings.
 - Weigh at same time each day, generally in morning after urinating, but before breakfast.
 - Use same scale.
 - Wear same type of clothing (eg, always in lightweight pajamas) or no clothing.
 - Keep daily weight diary.(77)
 - **EDUCATE:** Establish realistic weight goal.
 - **EDUCATE:** Maintain weight within narrow limits.[N](111)
 - **EDUCATE:** Importance of weight monitoring plan, and when and how to get help for weight changes
 - **COORDINATE:** Home care referral, if indicated
 - **COORDINATE:** Social worker referral, as needed(8)
 - **COORDINATE:** Medical equipment and supplies, if indicated
 - **COORDINATE:** Communication between care team members and care plan reconciliation(12)(14)(15)
 - **COORDINATE:** Follow-up contact to evaluate weight monitoring understanding, referral status, and need for further assistance(16)(17)
 - **SEND:** Weight diary (Illustrated)  (Spanish)
 - **SEND:** Weight changes and action plan  (Spanish)

Evidence Summary

Quality standards: The Case Management Society of America's Standards of Practice for Case Management principles include facilitation of informed decision making, counseling, and health education with patient self-care that is responsive to the patient's preferences (eg, culture, values, needs).(112)

Quality standards: Standards indicate that program content should include an individualized self-care plan that includes patient preferences, prioritized goals, interventions with information on whom to contact in urgent situations, and scheduled follow-up. Programs should facilitate communication and collaboration with practitioners in a timely manner.(112)(113)(114)(115)

National guidelines: The 2018 Physical Activity Guidelines Advisory Committee advises that adults with chronic conditions or disabilities, who are able, should follow the key activity guidelines for adults. The guidelines recommend that adults should do at least 150 to 300 minutes a week of moderate-intensity or 75 to 150 minutes a week of vigorous-intensity aerobic physical activity, or an equivalent combination of moderate-intensity and vigorous-intensity aerobic activity. They should also do muscle-strengthening activities on 2 or more days a week. Older adults should do multicomponent physical activity that includes balance training as well as aerobic and muscle-strengthening activities.(116)

Quality standards: Standards indicate that programs should ensure that during care transitions the care plan is re-evaluated, modified, and communicated to appropriate care team members (eg, providers, caregivers) as needed to support the care transition.(112)(113)(114)(115)

Transition consideration: Several studies demonstrated a reduction in 30-day hospital readmissions when the patient follow-up was within 7 days of hospital discharge.(117)(118)(119) Earlier follow-up (eg, within 48 hours after hospital discharge) may also help reduce hospital readmissions for high-risk or vulnerable patients.(16) Follow-up appointment attendance rates were higher when appointments were scheduled before hospital discharge.(120)(121)

Outcomes: Several studies support the role of the pharmacist in medication reconciliation to identify and resolve pharmacy-related problems and improve outcomes (eg, prevention of medication-related adverse events, discontinuation of inappropriate medication).(122)

Evidence: A systematic review that included 31 trials of 1817 women found with confidence that pelvic floor muscle training (PFMT) can cure or improve stress urinary incontinence (SUI) symptoms and should be recommended in management programs.(123)

National guidelines: The U.S. Preventive Services Task Force recommends exercise interventions (eg, gait training, balance training) to help prevent falls in at-risk adults age 65 years or older.(95)

National guidelines: The U.S. Preventive Services Task Force recommends exercise-based interventions to help reduce falls for people 65 years and older. Other interventions (eg, behavioral therapy, nutrition therapy, environmental modification, medication changes) may be considered based on the patient's preferences, the benefit-to-harm balance, and the circumstances of prior falls. Vitamin D supplementation is not recommended for fall prevention for older people without a known deficiency or osteoporosis.(95)

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Footnotes

[A] Assess and address the emergency or urgent problems before continuing the engagement. [A in Context Link 1]

[B] A self-care plan tells you how to take care of your condition and when and where to get help for problems. [B in Context Link 1]

[C] Children with a history of short bowel syndrome often have aversions to eating. Referral to a speech-language pathologist and occupational therapist will begin a structured feeding plan progressing to age-appropriate foods.(124) [C in Context Link 1, 2]

[D] Thorough cooking means cooking meat at a high enough temperature for long enough to kill any foodborne bacteria. Techniques to ensure meat is cooked thoroughly include cooking meat until no blood juice comes out when it is poked with a fork and using a meat thermometer to determine if the food is cooked enough. Beef should be cooked to 160 degrees F (71.2 degrees C), pork to 160 degrees F (71.2 degrees C), and poultry to 165 degrees F (73.9 degrees C).(42) [D in Context Link 1]

[E] The timing of provider follow-up depends on the patient's disease severity, self-care complexity, patient/caregiver willingness and ability to provide patient care, and available social supports.(125) [E in Context Link 1]

[F] In the transplant populations, sarcopenia may present as low serum albumin, prealbumin, or cholesterol; BMI less than 23; unintentional weight loss; muscle wasting; and low protein and/or energy intake. Patients may already be experiencing sarcopenia due to end-stage renal disease, cirrhosis, heart failure, or COPD prior to transplant.(126) [F in Context Link 1]

[G] There are many types of immunosuppressives. See a drug reference for specific information about immunosuppressives not listed here. [G in Context Link 1]

[H] Weight loss can be effective treatment for urinary incontinence.(80) [H in Context Link 1]

[I] Patients may have up to a 10-point difference in readings between their two arms. For ongoing monitoring, blood pressure should be taken in the arm with the higher reading.(82) [I in Context Link 1]

[J] The difference in readings may be associated with subclavian artery disease, supraaortic stenosis, aortic coarctation, or aortic dissection.(85) [J in Context Link 1]

[K] A pharmacist can support medication reconciliation for redundancy or duplication and identify drug interactions (eg, with other drugs, food).(86) [K in Context Link 1]

[L] Services to consider for community support include stairway modifications, widening of interior doors for ambulation equipment, and installation of handrails.(106) [L in Context Link 1]

[M] Many driver rehabilitation programs provide therapy and behind-the-wheel assessments. Therapy assessment covers motor, cognitive, perception, and reaction times. The behind-the-wheel assessment covers fitness to drive.(127) [M in Context Link 1]

[N] Instruction on reporting weight changes depends on the patient's medical condition. Generally, a body weight loss of 5% or more in 6 months is an indicator for further assessment.(111) [N in Context Link 1]

Codes

ICD-10 Diagnosis: D89.811, D89.813, J44.81, J4A.0, J4A.8, J4A.9, T86.00, T86.01, T86.02, T86.03, T86.09, T86.10, T86.11, T86.12, T86.13, T86.19, T86.20, T86.21, T86.22, T86.23, T86.290, T86.298, T86.30, T86.31, T86.32, T86.33, T86.39, T86.40, T86.41, T86.42, T86.43, T86.49, T86.5, T86.810, T86.811, T86.812, T86.818, T86.819, T86.8401, T86.8402, T86.8403, T86.8409, T86.8411, T86.8412, T86.8413, T86.8419, T86.8421, T86.8422, T86.8423, T86.8429, T86.8481, T86.8482, T86.8483, T86.8489, T86.8491, T86.8492, T86.8493, T86.8499, T86.850, T86.851, T86.852, T86.858, T86.859, T86.890, T86.891, T86.892, T86.898, T86.899, T86.90, T86.91, T86.92, T86.93, T86.99 [Hide]

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